SAATHBAN

FOR ELDERLY CARE · PAKISTAN

AGEING IN A YOUNG NATION

A Secondary Report on Global and Pakistan Demographics with Special Emphasis on the Elderly

First Draft

Findings · Policy Landscape · Institutional Ecosystem

SAATHBAN · LAHORE

APRIL 2026

SAATHBAN

Timeless Togetherness

Ageing in a Young Nation

A Secondary Report on Global and Pakistan Demographics with Special Emphasis on the Elderly

First Draft

April 2026

Prepared by the Saathban Research Unit

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Contact

Saathban for Elderly Care
Email: admin@saathban.com
Web: www.saathban.com

FOREWORD

Founders' Remarks

We have spent centuries trying to understand what human beings are. One common phenomenon among all of the ways we are defined is that we "think" with complexity, thanks to the billions of neurons that make trillions of synapses, giving our brain a processing capacity that rivals many supercomputers combined. The other defining trait is our ability to socialize, so much so that we are characterized as "social animals". In essence, these are not very separate phenomena. For example, the prefrontal cortex, the part that makes us capable of empathy, of planning, of meaning, grew because social life demanded it, but that is a separate discussion. The interesting question, however, is if that latter trait starts diminishing (if there is no socialization) how will we define "human beings"? Are we going to change the definition of human beings, or are we going to work toward changing the circumstances that are keeping us from this very fundamental need?

With the advent of new technology, that distance has only increased, and we have become progressively less social. Loneliness is the new epidemic, which has, like many pandemics before it, affected our elderly the most. With the population of people above 60 approaching nearly two billion by 2050, the need to address loneliness has never been greater. And when we call loneliness a pandemic, we are not doing so for the sake of rhetorical flourish. Research shows it is far worse than most imagined, linking it to Alzheimer's, obesity, cognitive decline, and mortality rates that rival heavy smoking. As per John Cacioppo from the University of Chicago, who is one of the most revered authorities in this field, loneliness rewires your stress response, inflames your body, and shortens your telomeres. It kills quietly, with the efficiency of a chronic disease, because that is exactly what it is. And we have my deepest regards for Cacioppo's research on understanding this very phenomenon of the human brain, which is why it was just to mention him.

With that scenario, the idea of doing something to tackle it matters deeply. But where to begin? Many times in our lives, there is a surge of emotion where we want to do something for others. It is a normal thing. The upheaval of those emotions and the settling of them has always been with us, and it is something that is common. At least temporarily, these emotions lead us toward something good. But often, the way we end up helping in the spiral of this dopamine rush isn't very efficient. The question is whether helping is the goal or helping well is the objective. However you define efficiency, one thing remains non-negotiable. The fundamental background context of the people you are engaging with is an absolute prerequisite. Whatever you decide after that is a separate debate.

This report is an effort to present a glimpse of the current picture of the elderly globally, with a special emphasis on Pakistan. Many times, when we try to understand something, there is just so much information and data that we drown in it. This report tries to do the opposite, presenting what is most useful in a way that any person can follow. It presents the demographic realities and current frameworks for the elderly in Pakistan in a very sublime manner. It is especially useful for students, policymakers, and those working in sociology, law, human development, and health.

We have put enormous effort and sincerity into this. This is our little gift to the elderly of Pakistan, with many more to come.

Maheen Shafiq **Tahir Sajeel Farooq**

Co-Founders @ Saathban

FRONT MATTER

Acknowledgements

This secondary report has been compiled by the Saathban Research Unit as part of the organization's broader programme of evidence-building on population ageing and elderly welfare in Pakistan. The Unit is grateful to the United Nations Department of Economic and Social Affairs, Population Division, for the World Population Prospects 2024 dataset that underpins the demographic analysis presented in Sections 2 and 3. We further acknowledge the Pakistan Bureau of Statistics for the Digital Census 2023, which provides the empirical foundation of Section 4.

We extend our thanks to the government departments, charitable foundations, advocacy bodies, medical institutions and private care providers whose publicly available information on their services to older persons has made the institutional mapping in Section 6 possible. Responsibility for any error of fact or interpretation rests solely with the Research Unit.

This is a first-draft working document. A revised and expanded edition will be issued following consultation with Saathban's founders, partner organizations, and external reviewers.

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EXECUTIVE SUMMARY

Main Findings at a Glance

The world is ageing, and Pakistan is not the exception it is often assumed to be. This report brings together two strands of evidence that have so far been considered separately in the Pakistani policy conversation: the global demographic transition that is now in advanced stages in high-income societies, and the parallel but earlier-stage transition that is already under way within Pakistan itself. The picture that emerges is more urgent than the conventional narrative of a young country with a demographic dividend allows.

The Global Picture

Between 1950 and 2023, the global elderly population grew to 6.4 times its 1950 size, at approximately twice the rate of overall population growth. The 65-plus share of the world's population nearly tripled, from around five per cent to ten per cent, with acceleration visible from 2000 onward. The old-age dependency ratio doubled from roughly 0.08 to 0.16 over the period, and from 2010 onwards the continued rise in elderly dependency can no longer be offset by falling youth dependency. For public finance systems, this represents an embedded structural pressure that will persist for several decades.

Pakistan in Comparative Perspective

Pakistan sits at an earlier but dynamic stage of the same transition. Total population rose more than sixfold between 1950 and 2023, from approximately 37 million to around 235 million. Within that expansion, the elderly cohort grew to roughly sixteen times its 1950 size, a rate of internal acceleration more than double what the global aggregate recorded over the same period. Pakistan's 65-plus share, at approximately four to five per cent in 2023, remains far below the global mean, but the direction of change is unambiguous and the pace is quickening as the very large birth cohorts of the 1970s and 1980s approach retirement.

The 2023 Census in Focus

The Pakistan Digital Census 2023 records a national population of 241.5 million, of whom 13.52 million, or 5.62 per cent, are aged 60 and above. This cohort is male-dominated in all four elderly age bands, a pattern that departs from international norms and reflects historically elevated female mortality. Punjab alone holds approximately 5.11 million elderly persons, the single largest provincial concentration in the country, while Islamabad records the highest elderly share at 4.31 per cent. Balochistan, with only 0.31 million elderly persons and a 65-plus share of 2.11 per cent, retains the youngest demographic profile of any province.

The Policy and Legal Architecture

Pakistan's legal framework for the protection of elderly citizens has evolved substantially since the late 1990s, but it remains uneven. Four provinces and the Islamabad Capital Territory now have dedicated senior-citizens statutes. The Punjab Senior Citizens Welfare Act 2026, passed in February 2026, closes the last major provincial legislative gap. However, its age threshold of seventy years, as against the sixty-year standard used elsewhere, creates an anomaly that warrants immediate attention. Enforcement deficits, limited budgetary commitments and restricted participation of older persons in policy design remain systemic weaknesses across the framework as a whole.

The Institutional Ecosystem

Formal elderly care in Pakistan is delivered by a modest but varied set of organisations. This report documents nineteen such institutions, grouped into four sectors: a state safety net anchored in the Punjab Social Welfare Department, Pakistan Bait-ul-Maal and the Ehsaas Panagah programme; large charitable foundations led by Edhi, Alkhidmat, Chhipa, JDC and the Bahria Town home; smaller and faith-based providers; and a newer private tier of residential and home-based services concentrated in Lahore. Karachi and Lahore hold most of the provision. Outside these corridors, formal elderly care is thinly documented.

KEY TAKEAWAY

Pakistan retains a youth-heavy demographic structure, but the mechanisms that produced ageing in the rest of the world are now clearly operating within it. The country has a narrowing policy window in which the falling youth dependency ratio creates fiscal space and a productive surplus that could, if directed toward elderly-support infrastructure, cushion the old-age burden that cohort momentum will generate in the decades ahead. The evidence compiled in this report is intended to inform that redirection.

SECTION 1

Introduction and Scope

Saathban was established to address a question that is easy to ignore in a country whose public conversation is dominated by youth, employment and political flux: what happens to Pakistan's older citizens, and what will happen to them as their numbers grow? This first Secondary Report is the Foundation's opening attempt to assemble, in a single volume, the demographic, legal and institutional evidence that such a conversation requires.

The report is described as 'secondary' because it does not present new primary research. Its purpose is to consolidate the existing evidence base — from international datasets, national census records, statutory instruments and institutional records — into a coherent analytical narrative. A subsequent Primary Report will present Saathban's own field research, the results of surveys among older Pakistanis, and direct engagement with the institutions documented here.

1.1 Structure of the Report

The report is organised into five substantive sections following this introduction. Section 2 establishes the global demographic baseline, drawing on UN World Population Prospects 2024 data to document how the world's age structure has shifted since 1950. Section 3 narrows the lens to Pakistan, applying the same framework to trace the country's trajectory over the same period and to locate Pakistan within the global transition. Section 4 draws on the Pakistan Digital Census 2023 to provide a granular snapshot of the current population structure and, in particular, of the 13.52 million Pakistanis aged sixty and above.

Section 5 then moves from demography to policy, surveying the international instruments to which Pakistan is a signatory, the federal legislative and policy developments in the welfare-of-elderly space, and the dedicated provincial statutes enacted in Khyber Pakhtunkhwa, Sindh, Balochistan, Islamabad Capital Territory and, most recently, Punjab. Section 6 concludes with an institutional mapping of the nineteen organisations that currently deliver formal elderly-care services in Pakistan, grouped by sector and with a complete directory provided as an appendix.

1.2 Data Sources

The demographic analysis in Sections 2 and 3 draws on the 2024 revision of UN World Population Prospects, the most recent global dataset available at the time of writing. The census analysis in Section 4 uses the Pakistan Digital Census 2023 conducted by the Pakistan Bureau of Statistics, the first full national enumeration since 2017 and only the sixth in Pakistan's post-independence history. The legal analysis in Section 5 relies on primary statutory texts supplemented by government gazettes and publicly available legislative records. The institutional mapping in

Section 6 relies on information published by the organisations themselves and on publicly available secondary sources.

1.3 A Note on What This Report Does Not Do

This is a compilation and consolidation exercise. It does not attempt to forecast future demographic trajectories, to model the fiscal cost of an ageing population, to evaluate the performance of any individual institution, or to assess the lived experience of older Pakistanis. These are all legitimate and urgent questions, and Saathban intends to address them in subsequent publications. What this report does do is establish the factual terrain on which those further enquiries can rest.

> **A FIRST DRAFT**
>
> This is explicitly a first-draft working document, prepared for publication on the Saathban website and intended to invite comment, correction and suggestion. Several sections remain to be added, including a dedicated treatment of the lived experience of older Pakistanis, a policy-recommendation chapter, and an expanded set of data visualisations. Readers are encouraged to write to the Foundation with observations that can be incorporated in the revised edition.

SECTION 2

Global Demographic Trends

Drawing on UN World Population Prospects (2024) data spanning 1950 to 2023, this section establishes the demographic foundations of the broader study. It documents the structural shift in global age composition that underlies the challenges examined in subsequent sections.

2.1 Overall Population Growth and Cohort Structure

Global population rose from approximately 2.5 billion in 1950 to over 8 billion by 2023, a more than threefold increase across seven decades. The working-age cohort, aged 25 to 64, was the dominant driver of this expansion, growing from roughly 1.1 billion to approximately 4.7 billion over the same period. It is this cohort's growth that sustained global economic output, tax revenues and social-transfer systems during the latter half of the twentieth century.

The elderly population, aged 65 and above, though visually modest in aggregate charts, tells a fundamentally different story. From an estimated 130 million in 1950, this cohort reached approximately 770 to 800 million by 2023. Meanwhile, the under-five cohort has plateaued and slightly contracted in relative terms as global fertility rates have declined. This divergence between a shrinking base of young children and a rapidly expanding elderly cohort represents the core structural fact driving the concerns examined throughout this research.

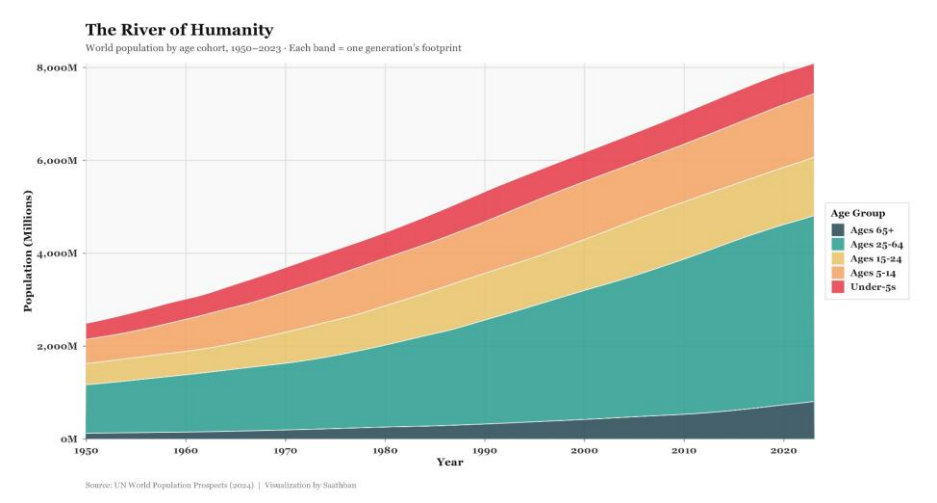

Figure 2.1: The River of Humanity: World population by age cohort, 1950–2023

2.2 Proportional Shifts in Global Age Composition

Examining population shares rather than absolute numbers makes the structural transformation more legible. In 1950, persons under twenty-five collectively accounted for roughly 52 to 54 per cent of the world's population, with children aged under five alone comprising approximately 14 to 15 per cent. By 2023, the under-five share had declined to around seven per cent, and younger cohorts broadly had ceded proportional ground to older age groups.

The 65-plus share, which stood at approximately five per cent in 1950, had risen to roughly ten per cent by 2023. This near-tripling of the elderly share over seven decades has no historical precedent. The working-age cohort did see its share expand from about 46 per cent in 1950 to nearly 59 per cent by 2023, yielding what economists term a demographic dividend. However, as cohorts born in the mid-twentieth century move into retirement, this dividend is giving way to a demographic liability.

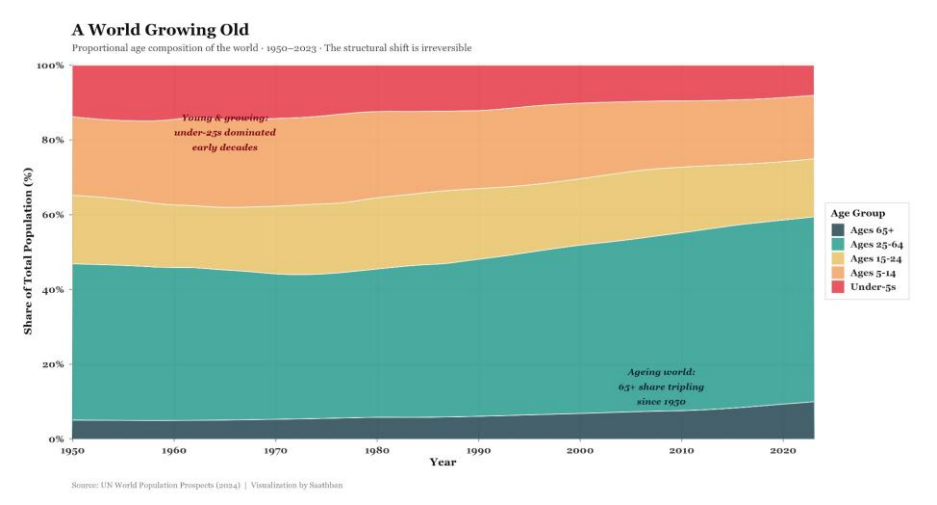

Figure 2.2: A World Growing Old: Proportional age composition of the world, 1950–2023

2.3 Comparative Cohort Growth Rates

Indexing all cohorts to a baseline of 100 in 1950 isolates their comparative growth velocity. By 2023, the elderly population had reached an indexed value of 640, meaning it grew to 6.4 times its 1950 size, while global population as a whole grew to approximately 3.2 times. The elderly cohort therefore grew at roughly twice the rate of overall population. For comparison, the working-age cohort reached an index of 384, the 15-to-24 and 5-to-14 cohorts clustered around 261 to 275, and the under-five cohort reached only 190.

Three mechanisms account for this outcome: mortality compression through improved medicine and sanitation; cohort size, as the large birth cohorts of the 1950s and 1960s flow into the 65-plus

bracket; and the relative decline in younger cohorts, which means the elderly share grows both because the numerator rises and the overall denominator grows more slowly. These mechanisms compound one another, which explains why the growth divergence is not merely large but accelerating.

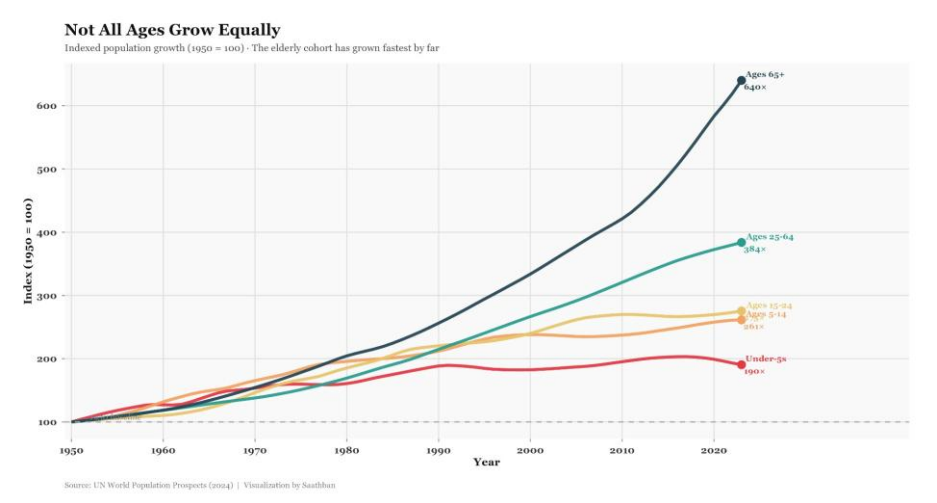

Figure 2.3: Not All Ages Grow Equally: Indexed population growth by cohort (1950 = 100)

2.4 Dependency Ratios and Fiscal Implications

Translating these trends into an economic framework, the old-age dependency ratio, defined as elderly persons per working-age adult, rose from approximately 0.08 in 1950 to 0.16 by 2023. Over the same period, the youth dependency ratio fell from a peak of around 0.67 in the mid-1960s to approximately 0.39 by 2023, producing a net reduction in total dependency that underpinned the demographic dividend. However, from around 2010 onward, falling youth dependency can no longer offset rising elderly dependency, and total dependency has begun to edge upward again.

The fiscal implications are considerable. Youth dependency entails expenditure primarily on education and basic healthcare, which are broadly time-limited investments. Elderly dependency involves sustained expenditure on healthcare, which rises steeply with age, on long-term care, and on pension transfers. These are recurring and growing costs that cannot be easily reversed: immigration at the scale required to offset ageing in large economies is politically and logistically constrained, and no other mechanism is available to reduce the ratio within a policy-relevant timeframe. The trajectory therefore represents an embedded structural pressure on public finance systems that will persist for several decades. It should be noted that the figures here are global aggregates masking substantial regional variation, and should be read as indicative of direction and magnitude rather than as directly applicable to any specific national context.

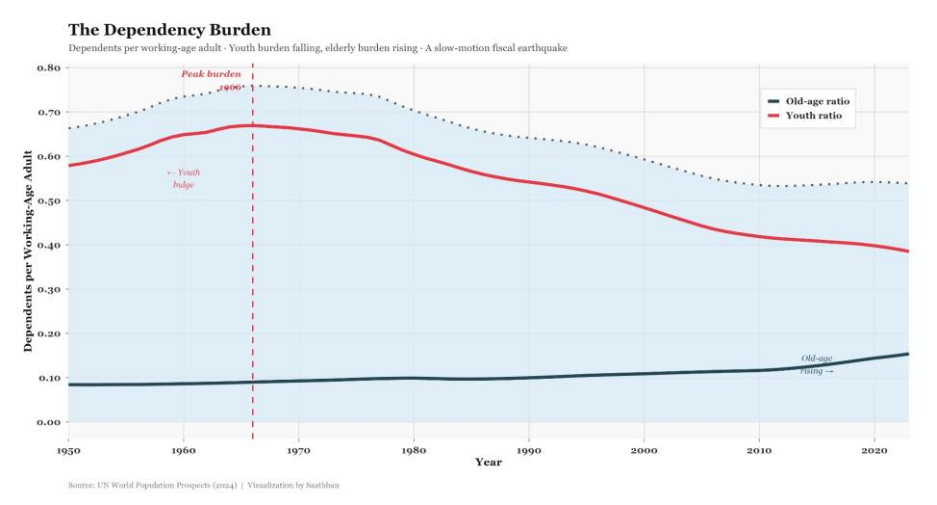

Figure 2.4: The Dependency Burden: Dependents per working-age adult, 1950–2023

2.5 Section Summary

The evidence from this section establishes that the world's elderly population grew to 6.4 times its 1950 size by 2023, at approximately twice the rate of overall population growth. The elderly share of global population nearly tripled over the same period, from roughly five per cent to ten per cent, with acceleration visible from 2000 onward. The working-age demographic dividend that temporarily cushioned total dependency ratios is now transitioning as large mid-century cohorts move into retirement. The old-age dependency ratio will continue to rise for decades due to cohort momentum, extended longevity and structural fertility decline. These trends frame the policy and institutional challenges examined in the remainder of this research.

Source: *UN World Population Prospects (2024).*

SECTION 3

Pakistan Demographic Trends

Drawing on the same UN World Population Prospects (2024) dataset used in Section 2, this section examines Pakistan's age-structure trajectory from 1950 to 2023. Pakistan presents a demographically distinct case from the global aggregate: it remains a predominantly young society, yet the structural forces that produced ageing at the global level are now operating within it, albeit at an earlier stage and with greater intensity in specific cohorts. Understanding this trajectory is essential because the pressures it generates differ in character from those documented at the global level, and because the policy window for managing them is narrowing.

3.1 Overall Population Growth and Cohort Structure

Pakistan's total population rose from approximately 37 million in 1950 to around 231 to 235 million by 2023, a more than sixfold increase across seven decades. This rate of expansion significantly exceeded the global threefold increase documented in Section 2, making Pakistan one of the most rapidly growing large populations in the recorded period. The working-age cohort, aged 25 to 64, was the dominant structural driver of this growth, expanding from roughly 13 million in 1950 to over 100 million by 2023. This cohort's expansion underpinned the labour-force growth that sustained Pakistan's economic activity across the period, though its productive potential has been partially constrained by structural weaknesses in employment, education and capital formation that lie beyond the scope of this section.

The elderly population, aged 65 and above, though still a modest share of the total, followed a trajectory of sharp absolute and relative acceleration. From approximately 648,000 persons in 1950, the cohort grew to an estimated 10 to 11 million by 2023. While this figure is small relative to the global elderly stock, its internal rate of growth, examined more closely in Section 3.3, is demographically significant. At the other end of the age spectrum, the under-five cohort displayed a pattern consistent with fertility transition: sustained growth through the high-fertility decades of the 1960s to 1990s, followed by a plateau and mild deceleration as total fertility rates declined. This divergence between a slowing young cohort and an accelerating elderly cohort represents the structural shift that defines Pakistan's current demographic moment.

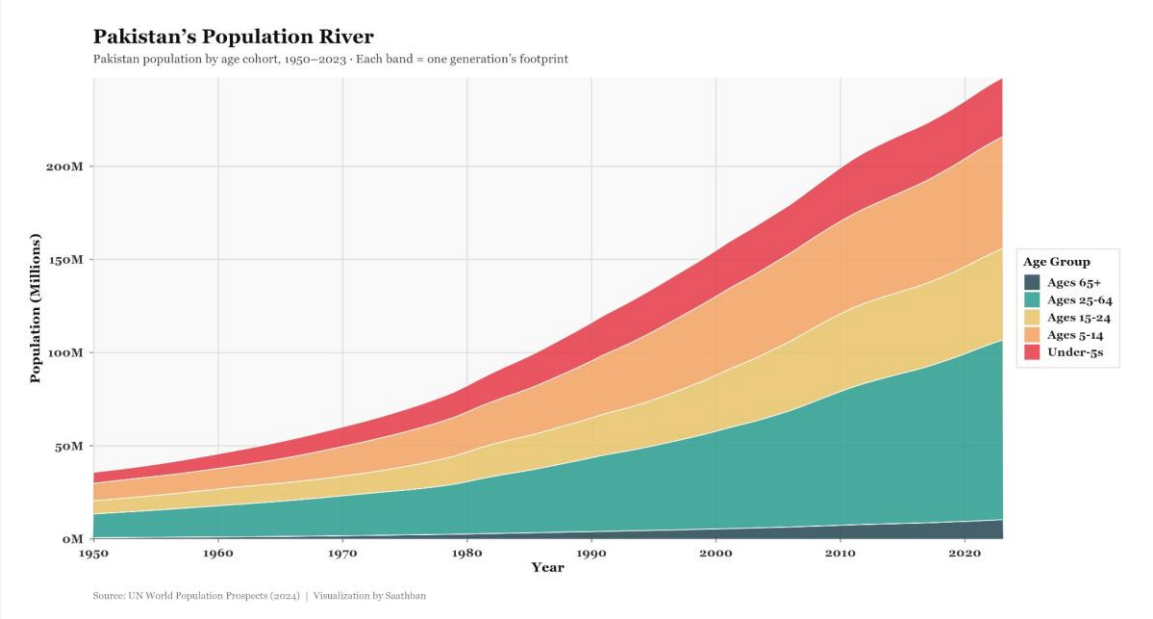

Figure 3.1: Pakistan's Population River: Population by age cohort, 1950–2023

3.2 Proportional Shifts in Age Composition

Examining population shares isolates the structural transformation more precisely than absolute numbers. In 1950, persons under twenty-five collectively accounted for approximately 60 to 62 per cent of Pakistan's population, with the under-five cohort alone comprising roughly 16 to 17 per cent. Pakistan was, in proportional terms, one of the youngest large populations in the world at that point. By 2023, the under-five share had declined to approximately nine to ten per cent, and younger cohorts broadly had ceded proportional ground to working-age adults. The 25-to-64 cohort's share rose from roughly 35 per cent in 1950 to approximately 43 per cent by 2023, a gain that reflects both sustained absolute growth and the relative slowing of younger cohorts.

The 65-plus share, which stood at under two per cent in 1950, had risen to approximately four to five per cent by 2023. This level remains far below the near ten per cent observed in the global aggregate and the much higher shares recorded in high-income ageing societies. However, the direction of change is unambiguous and the pace is accelerating. The proportional composition chart makes clear that while Pakistan retains a youth-heavy structure compared to the global average, the structural change visible in global data from 1980 onward is now detectable in Pakistan's own profile from roughly 2000 onward. The demographic dividend that the working-age share expansion should theoretically generate has not been fully realised, and as cohort momentum begins to push larger shares of the population into the 65-plus bracket in coming decades, the window for realising it is contracting.

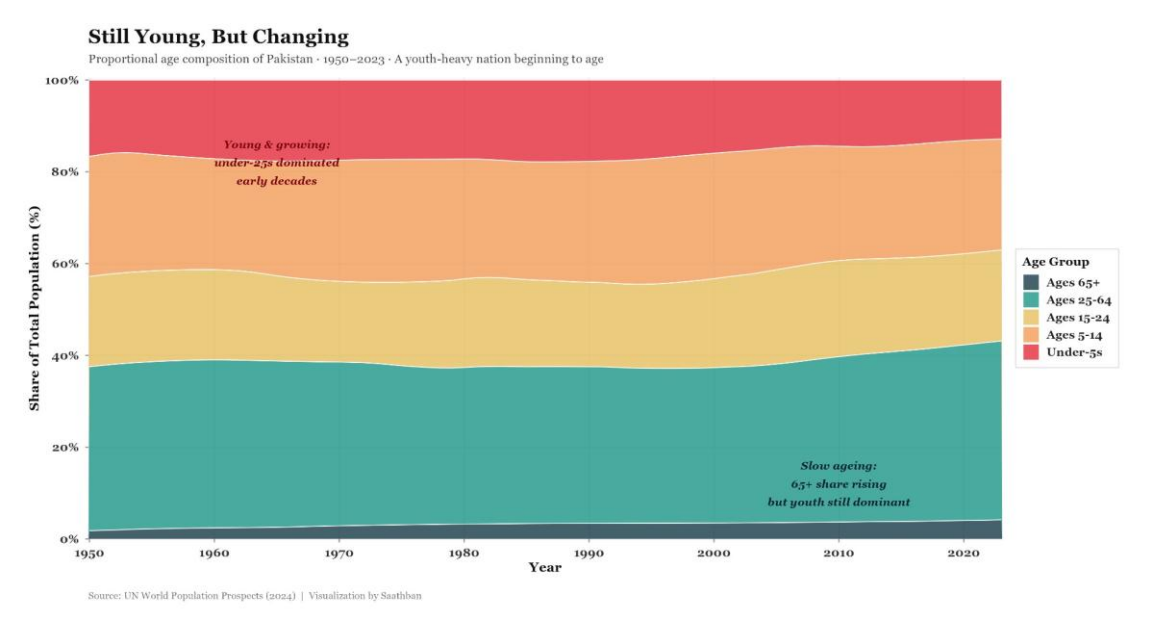

Figure 3.2: Still Young, But Changing: Proportional age composition of Pakistan, 1950–2023

3.3 Comparative Cohort Growth Rates

Indexing all cohorts to a baseline of 100 in 1950 reveals a pattern of divergence that is more extreme in Pakistan than in the global aggregate. By 2023, the elderly population reached an indexed value of approximately 1,600, meaning it grew to around sixteen times its 1950 size. The working-age cohort reached approximately 752, and the youth cohorts, aged 5 to 14 and 15 to 24, clustered in the range of 630 to 700. The under-five cohort, the slowest-growing, reached approximately 533, reflecting the fertility transition already noted.

The 1,600 indexed value for the 65-plus cohort stands in sharp contrast to the 640 recorded for the same cohort in the global aggregate examined in Section 2. Three compounding mechanisms explain this disparity. First, Pakistan's elderly population in 1950 was extremely small in absolute terms, so even modest absolute additions produced large proportional gains. Second, improvements in child and adult mortality across the period were substantial, extending survival into old age at an accelerating rate from the 1970s onward. Third, cohort momentum from the very large birth cohorts of the 1970s and 1980s is now flowing into the upper age brackets, producing an acceleration that will intensify through the 2030s and 2040s. The index chart makes the separation between the elderly trajectory and all other cohorts visually unmistakable: from roughly 2000 onward, the 65-plus line diverges sharply upward while all other cohorts continue on broadly parallel, more gradual paths.

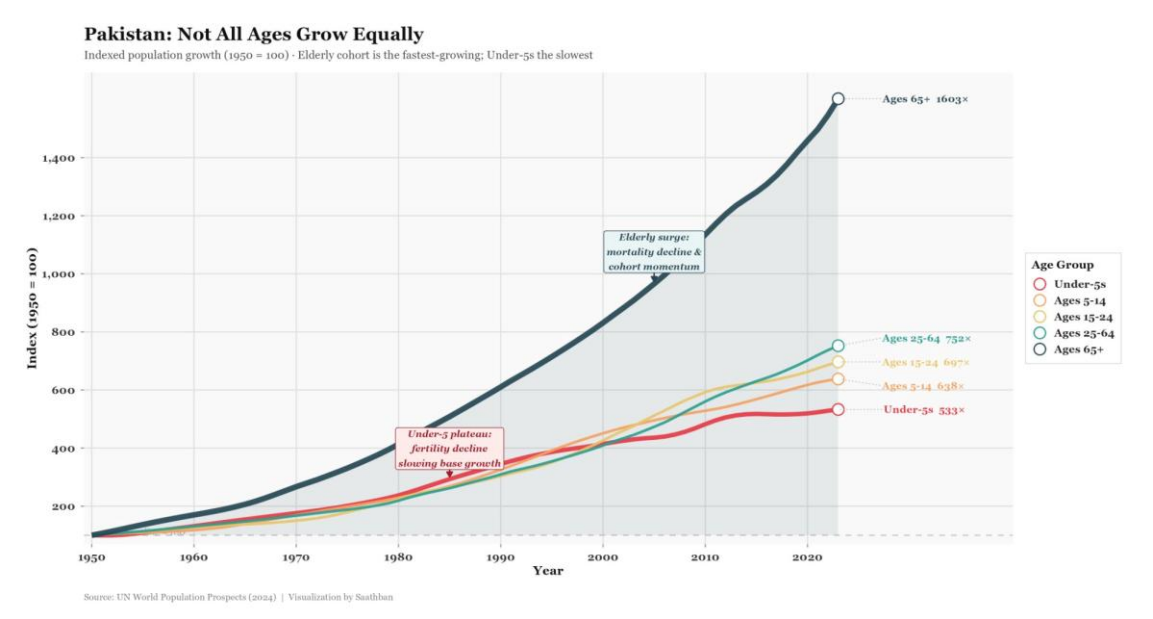

Figure 3.3: Not All Ages Grow Equally: Indexed population growth by cohort, 1950–2023 (1950 = 100)

3.4 Where Pakistan's Growth Landed: A Decade-by-Decade View

The decade-on-decade chart provides a complementary perspective to the index chart by showing, in absolute terms, which cohorts absorbed Pakistan's population growth in each ten-year interval. In the 1950s, total population added was approximately 9.9 million, distributed relatively evenly across cohorts in a pattern consistent with high fertility and high mortality. Through the 1960s and 1970s, as mortality declined while fertility remained high, total additions rose to 14.5 million and then 22.1 million per decade, with the working-age and younger cohorts capturing the largest absolute shares.

The acceleration continued sharply through the 1980s and 1990s, reaching 33.9 million and 38.7 million per decade respectively, with the 25-to-64 cohort becoming the single largest contributor to absolute additions from the 1980s onward. The peak decade in the data is 2000 to 2010, when Pakistan added approximately 44.4 million persons, with the working-age cohort contributing the largest portion. The 2010 to 2020 interval recorded slightly lower additions at 35.8 million, consistent with the early-stage fertility transition visible in declining under-five additions. Critically, across all decades, the 65-plus cohort remains a visually narrow band at the base of each bar, but its absolute contribution roughly doubled from one decade to the next after 1980, a pattern of acceleration that the stacked-bar format makes legible by decade even if the cohort's share remains small in proportional terms.

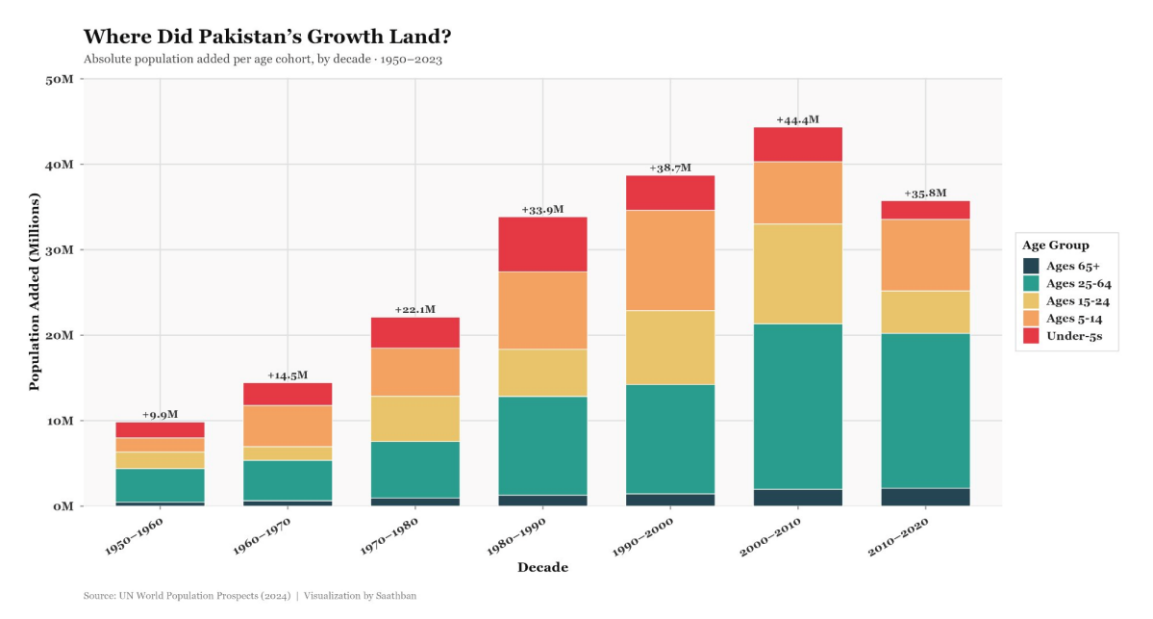

Figure 3.4: Where Did Pakistan's Growth Land? Absolute population added per age cohort, by decade, 1950–2023

3.5 Dependency Ratios and Emerging Fiscal Pressure

Translating these trends into an economic framework, Pakistan's dependency structure differs from the global pattern in both level and trajectory. The youth dependency ratio, or young dependents per working-age adult, was approximately 0.77 to 0.80 in 1950, substantially above the global figure of around 0.67 at the same date, reflecting Pakistan's higher fertility rates. This ratio rose through the 1960s and peaked around 1994, when it reached approximately 0.85 to 0.88, before declining as fertility rates fell. By 2023 it had retreated to approximately 0.63 to 0.65, still high by international standards but on a downward trajectory. The old-age dependency ratio, by contrast, remained very low throughout most of the period, rising from approximately 0.03 in 1950 to roughly 0.06 to 0.07 by 2023.

The total dependency ratio therefore remains dominated by its youth component, and the overall burden has declined from its 1994 peak. This is structurally different from the global picture presented in Section 2, where the youth and elderly burdens crossed and total dependency began rising again from roughly 2010. Pakistan has not yet reached that crossover. However, the direction of the old-age ratio is unambiguous and its pace is increasing. The fiscal implications of even a modest old-age dependency ratio are considerable in Pakistan's context, because the institutional infrastructure for elderly support remains underdeveloped relative to the scale of what will be required as the large cohorts of the 1970s and 1980s enter their seventies in the 2040s and 2050s. Declining youth dependency creates a fiscal dividend in the near term, but it also

reduces the working-age base available to finance future elderly support unless labour-force participation and productivity are substantially expanded in the intervening years.

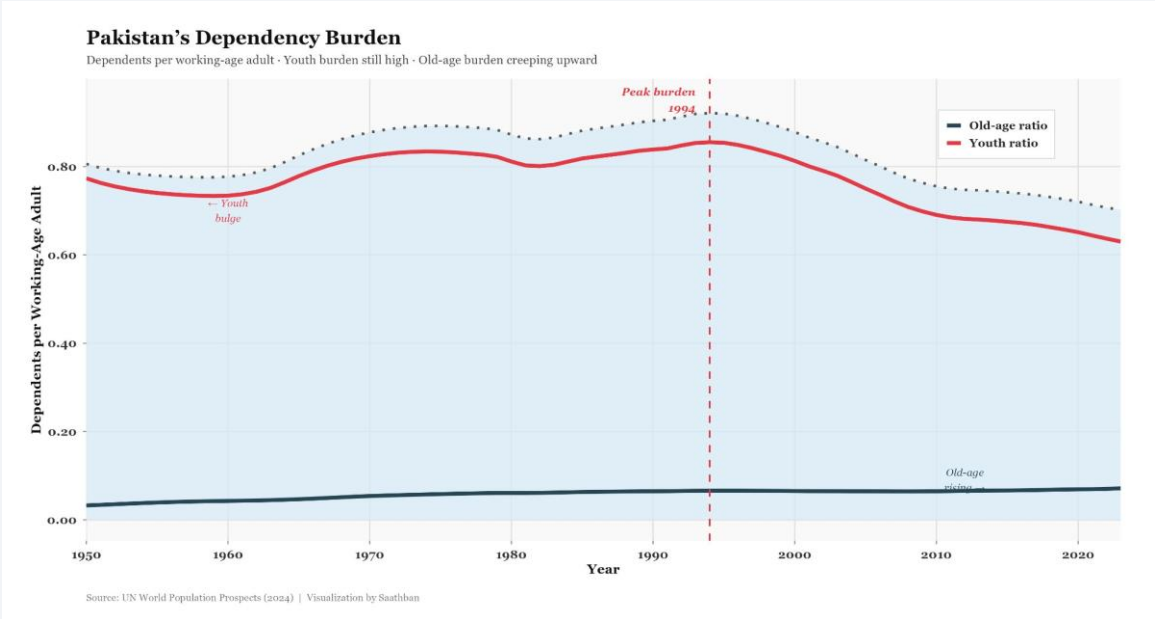

Figure 3.5: Pakistan's Dependency Burden: Dependents per working-age adult, 1950–2023

3.6 Section Summary

Pakistan's demographic trajectory from 1950 to 2023 is characterised by rapid and sustained total population growth, a strong but only partially realised working-age dividend, a fertility transition that is reducing youth dependency ratios from their 1994 peak, and an elderly cohort that, though small in proportional terms, has grown to approximately sixteen times its 1950 size and is accelerating. These features locate Pakistan at an earlier but dynamic stage of the same structural transition documented globally in Section 2. The country retains a youth-heavy profile that differs sharply from the ageing trajectories of high-income societies, but the mechanisms driving convergence toward older age structures are now clearly operating. The policy implication is that Pakistan faces a time-limited opportunity: the declining youth dependency ratio generates fiscal space and a productive surplus that, if directed toward health, education and institutional capacity for elderly support, could cushion the old-age burden that cohort momentum will produce in the decades ahead. The evidence from this section establishes that the structural shift is no longer a future possibility but an ongoing process, and that its demographic underpinnings are embedded in cohort sizes that cannot be reversed within any near-term policy horizon.

Source: *UN World Population Prospects (2024); author's calculations.*

SECTION 4

Pakistan Census 2023: Population Structure and the Ageing Cohort

This section draws on the Pakistan Digital Census 2023 conducted by the Pakistan Bureau of Statistics, the first full national enumeration since 2017 and only the sixth in Pakistan's post-independence history. It is divided into two analytical sub-sections. Sub-section 4.1 documents the overall demographic structure of Pakistan's 241.5 million population, examining provincial distribution, gender composition, the national age pyramid, provincial age profiles and the rural-urban balance. Sub-section 4.2 then narrows to a deep analysis of the population aged 60 and above, examining its absolute size, internal gender dynamics, sex-ratio trajectories across age bands, provincial variation in elderly shares and the structural position of each elderly cohort within the national total. Together these two sub-sections establish the empirical foundation for the policy and fiscal analysis that follows in subsequent sections of this research.

4.1 Overall Population Structure: Size, Distribution and Composition

4.1.1 Total Population and Provincial Distribution

Pakistan's total enumerated population in 2023 stands at 241.5 million, an increase of approximately 33.8 million from the 207.7 million recorded in the 2017 census. This represents a population growth of roughly 16.3 per cent over six years, corresponding to an average annual growth rate of 2.55 per cent. This rate is marginally higher than the 2.40 per cent recorded in the preceding inter-census period and places Pakistan among the fastest-growing large populations in the world, exceeded in absolute terms only by a small number of sub-Saharan African nations. Population density has risen correspondingly from 260.87 persons per square kilometre in 2017 to 303.35 in 2023, reflecting the intensifying pressure on land, infrastructure and public services.

The provincial distribution of this population has remained broadly stable across the two census periods. Punjab continues to dominate, with 127.69 million residents constituting 52.87 per cent of the national total, a marginal decline from 52.96 per cent in 2017. Sindh, the second most populous province, recorded 55.70 million persons or 23.06 per cent, effectively unchanged from its 2017 share of 23.04 per cent. Khyber Pakhtunkhwa including the merged districts of the former FATA recorded 40.86 million, or 16.92 per cent, a slight decline from 17.09 per cent, reflecting the faster growth of other regions. Balochistan, Pakistan's largest province by area but smallest by population, recorded 14.89 million persons, and its share rose modestly from 5.94 per cent to 6.17 per cent. Islamabad Capital Territory, the smallest administrative unit, recorded 2.36 million persons, or 0.98 per cent. These shifts are small in proportional terms but consequential for resource allocation formulas, National Finance Commission awards and parliamentary seat distribution, all of which are anchored to census-derived population figures.

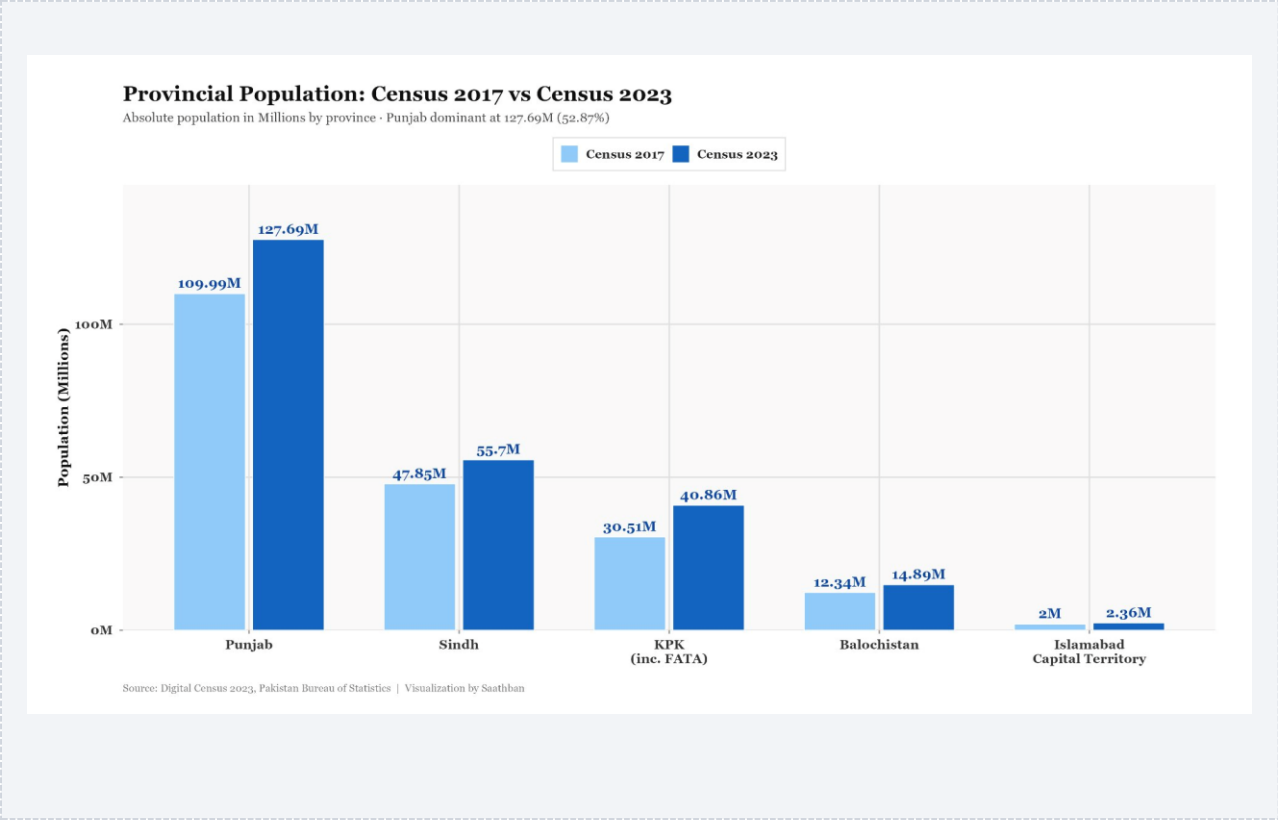

Figure 4.1: Provincial Population: Census 2017 vs Census 2023

4.1.2 Gender Composition

Pakistan's gender composition has remained highly stable across the two census periods. Males account for 124.32 million persons or 51.48 per cent of the population in 2023, compared with 106.32 million and 51.19 per cent in 2017. Females number 117.15 million, representing 48.51 per cent, up from 101.34 million in 2017. The transgender population remained at approximately 0.02 million across both enumerations. The sex ratio, expressed as males per 100 females, rose from 104.91 in 2017 to 106.12 in 2023. This elevation above the natural sex ratio at birth of approximately 105 reflects a combination of female undercount in conservative regions, differential mortality patterns and labour migration dynamics that remove working-age males from certain administrative units while they remain registered in their home districts. The absolute female population grew by 15.81 million between the two censuses, which is a faster absolute increment than in previous inter-census periods, consistent with improving female survival rates and declining maternal mortality over the period.

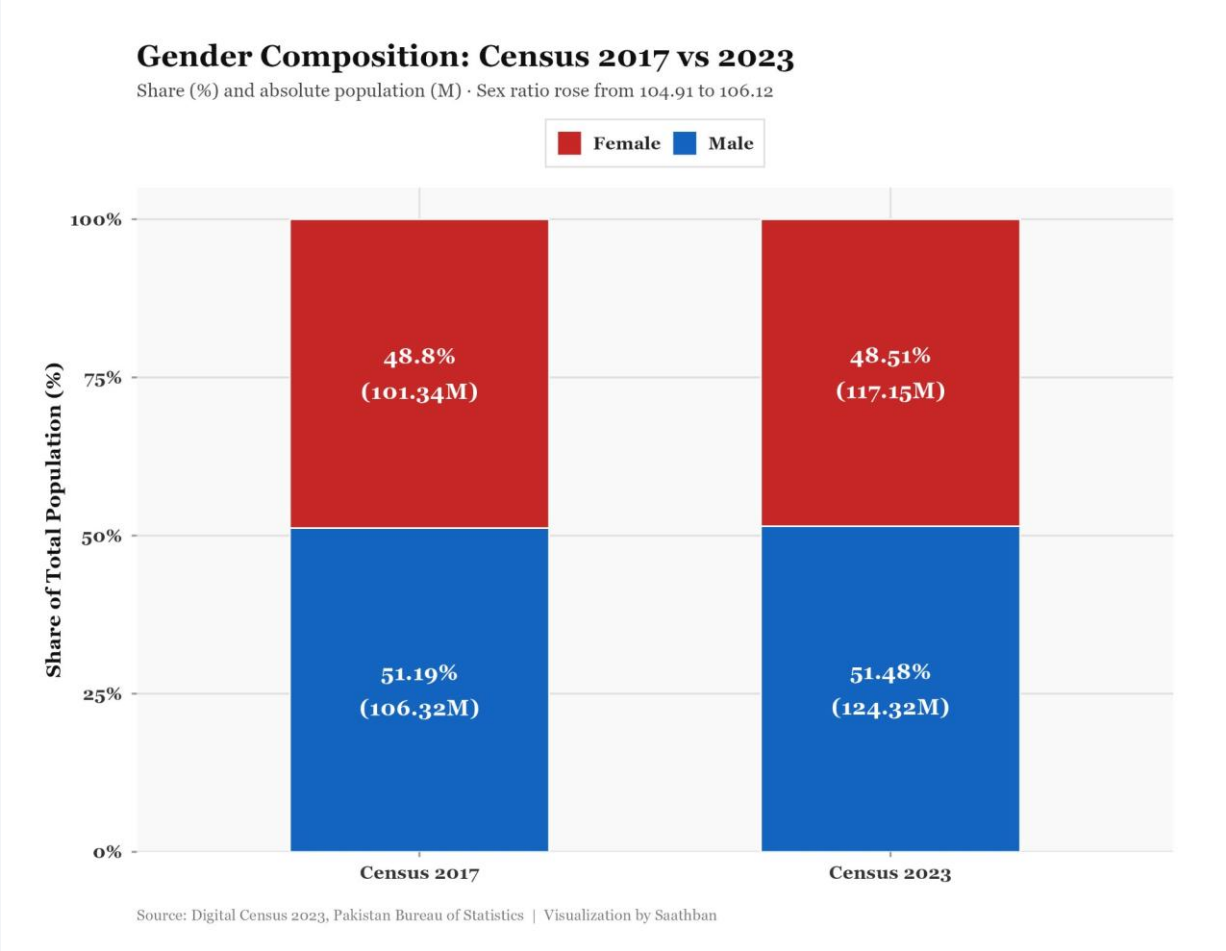

Figure 4.2: Gender Composition: Census 2017 vs Census 2023

4.1.3 Age Structure and the Population Pyramid

The national population pyramid for 2023 displays the classic broad-base, narrow-apex form associated with high-fertility, high-growth societies. The youngest cohort, aged 0 to 4, accounts for 15.2 per cent of the total population or 36.47 million persons, the single largest five-year band in the country. The five-year bands from 5 to 9 and 10 to 14 contribute a further 13.4 and 12.0 per cent respectively, meaning that nearly 41 per cent of Pakistan's population is below the age of fifteen. The 15 to 19 and 20 to 24 bands each contribute approximately ten and 8.5 per cent, producing a very large youth bulge in the 10-to-24 age range that carries substantial implications for education provision, labour-market absorption and social stability.

The pyramid narrows progressively but steeply from the working-age cohorts onward. The 25-to-64 age range, broadly representing the productive labour force, collectively constitutes approximately 44 per cent of the total population. The 55-to-59 band contributes 2.7 per cent, and

the 60-to-64 band 2.1 per cent, the first five-year band in which total share falls below 2.5 per cent. The bands above sixty progressively narrow to 1.5 per cent at 65 to 69, 1.0 per cent at 70 to 74, and 1.0 per cent at 75 and above, though the near-equality of the 70-to-74 and 75-plus bands is analytically notable and is examined further in sub-section 4.2. The overall structure is consistent with a demographic-dividend phase: a large and growing working-age base relative to the current elderly stock. However, the sheer size of the younger cohorts flowing toward working age, combined with the already-elevated sex ratio and the slow pace of female labour-force participation growth, means the dividend will be realised only partially unless labour-market and human-capital conditions change substantially.

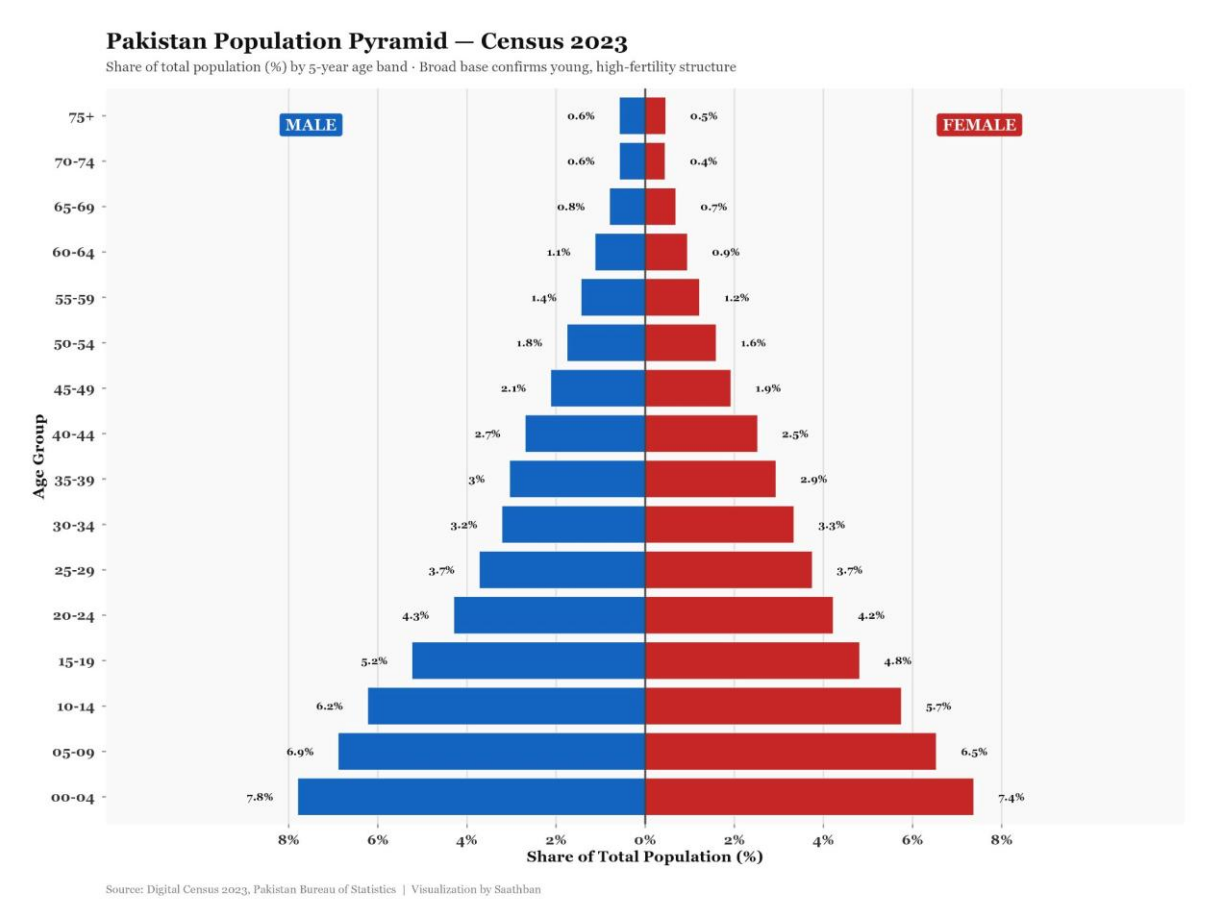

Figure 4.3: Pakistan Population Pyramid: Census 2023

4.1.4 Provincial Age Profiles

While the national pyramid presents a coherent aggregate picture, the provincial age profiles reveal marked internal heterogeneity. Balochistan displays the youngest demographic profile of any province: 49.66 per cent of its population is below fifteen years of age, and its 65-and-above

share stands at just 2.11 per cent. This combination of a very high child dependency ratio and a very low elderly share reflects persistently high total fertility rates, limited access to family planning services and elevated child mortality that historically inflates younger cohort sizes through high replacement-level births. KPK, including the former FATA, presents a similarly youthful profile, with 43.45 per cent below fifteen and 3.26 per cent aged sixty-five and above. Sindh falls in the middle range, with 41.78 per cent below fifteen and 3.06 per cent aged 65-plus.

Punjab and Islamabad Capital Territory present structurally older profiles relative to the national average, though both remain young by international standards. Punjab's under-fifteen share is 38.21 per cent, and its 65-plus share is 4.00 per cent. Islamabad exhibits the most advanced age profile, with only 32.28 per cent below fifteen and 4.31 per cent aged sixty-five and above, reflecting the concentrated presence of educated urban households, higher female labour-force participation, lower fertility and better health infrastructure that extends old-age survival. The 15-to-64 working-age share in Islamabad reaches 63.40 per cent, the highest of any province, compared with a national average below 55 per cent, reinforcing the capital territory's position as Pakistan's most structurally prepared administrative unit from a demographic-dividend standpoint.

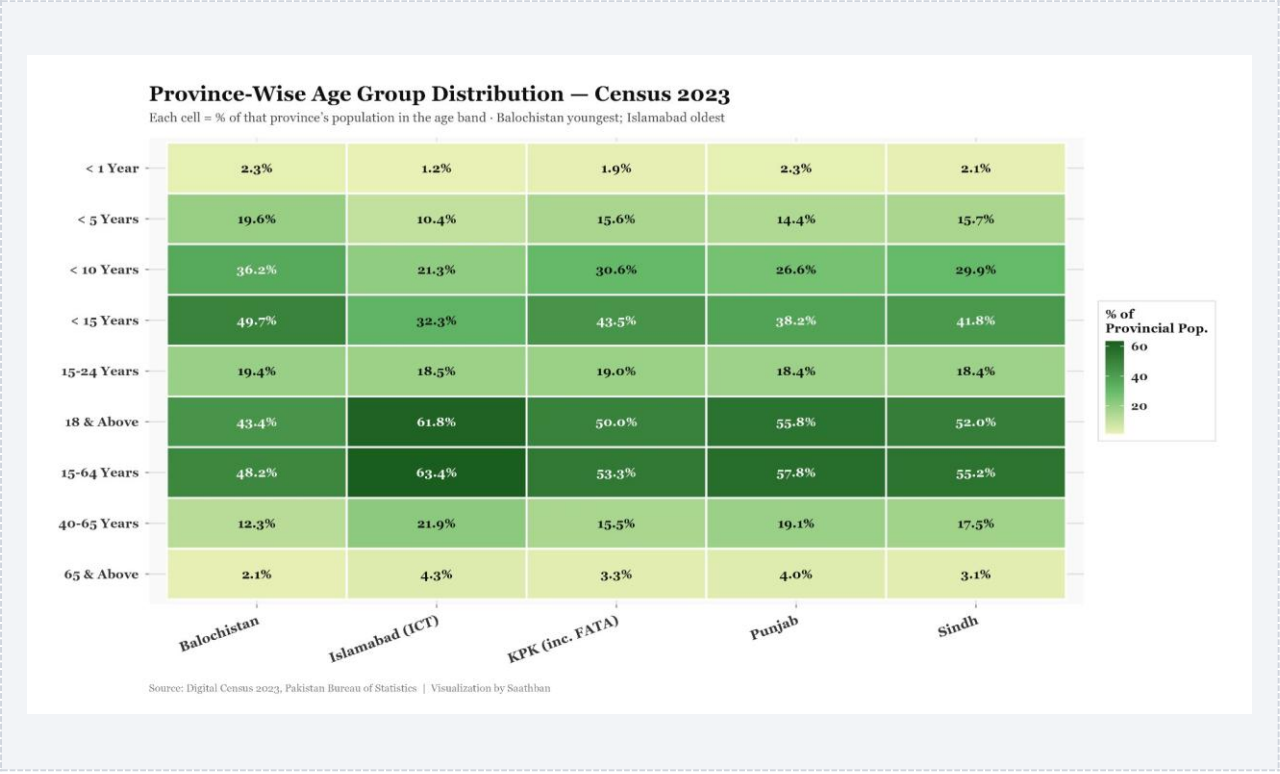

Figure 4.4: Province-Wise Age-Group Distribution: Census 2023

4.1.5 Rural and Urban Population

Pakistan remains a predominantly rural society, though the pace of urbanisation is accelerating. In 2023, 147.61 million persons or 61.12 per cent of the population reside in rural areas, compared with 132.01 million and 63.56 per cent in 2017. Urban population rose from 75.67 million (36.44

per cent) in 2017 to 93.88 million (38.88 per cent) in 2023, an absolute increase of 18.21 million urban residents over six years. This urban increment, equivalent to adding a city the size of greater Karachi to the urban stock in a single inter-census period, places enormous pressure on urban infrastructure, housing, transport, sanitation and social services. The 2.44-percentage-point shift in the rural-urban ratio over six years is among the largest in Pakistan's census history, suggesting that internal migration from rural to urban areas has intensified alongside natural urban population increase. The implications for the elderly population are particularly relevant: rural-to-urban migration tends to be selective of younger adults, leaving disproportionate shares of older persons in rural areas where healthcare and pension infrastructure are weakest.

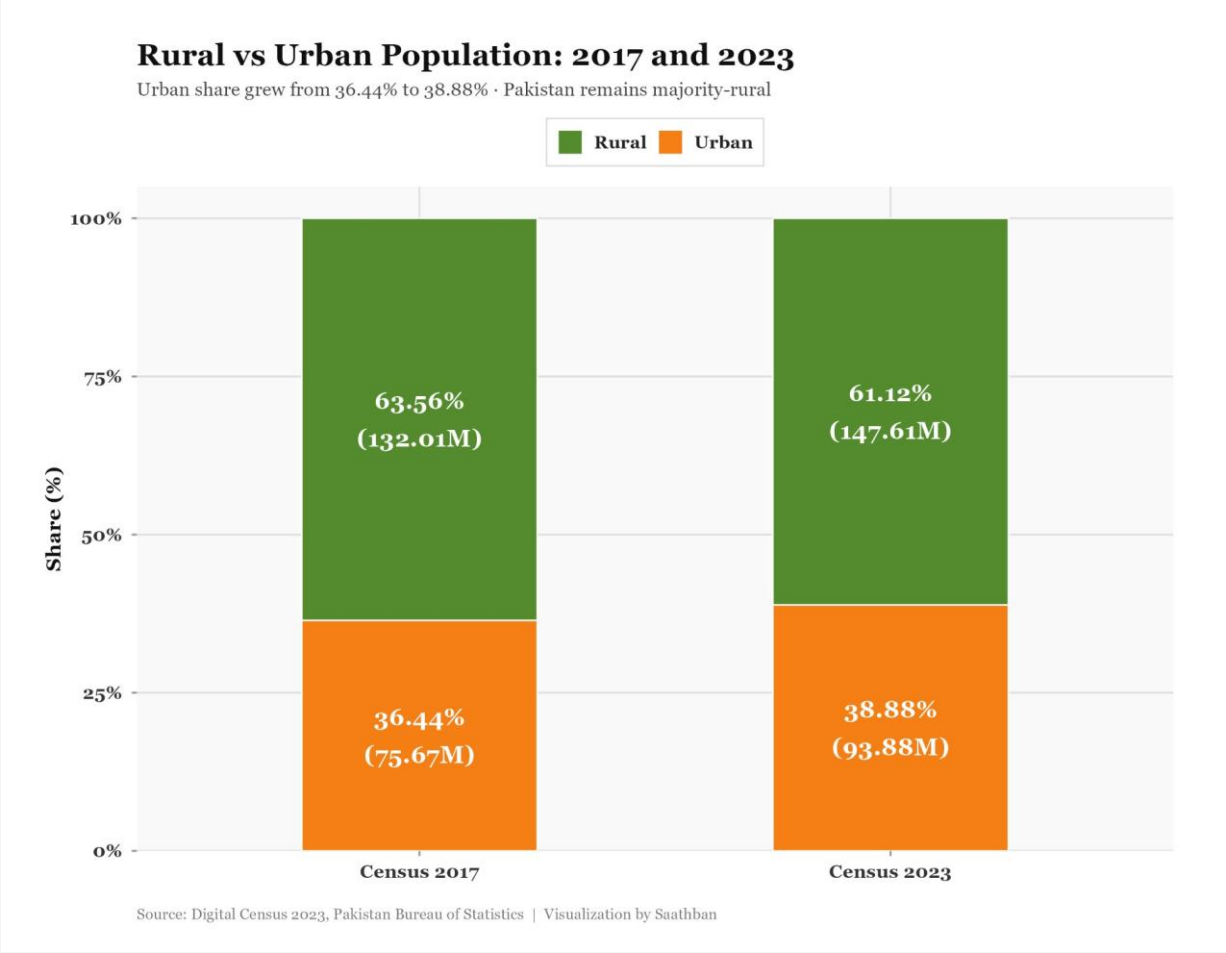

Figure 4.5: Rural vs Urban Population: 2017 and 2023

4.2 The Population Aged 60 and Above: A Deep Analysis

Sub-section 4.2 isolates the population aged 60 and above for detailed examination. This cohort is of particular policy relevance because it constitutes the primary beneficiary group for pension transfers, elderly healthcare provision and long-term care infrastructure. Pakistan has historically devoted limited institutional attention to ageing-related policy, operating under the assumption that traditional family structures absorb elderly care needs. The evidence from the 2023 census provides the most precise snapshot yet of how large this cohort has become, how it is distributed across gender and province and what its internal structure implies for the coming decades.

4.2.1 Size and Gender Breakdown of the 60-Plus Cohort

Pakistan's population aged 60 and above totals approximately 13.52 million persons as of the 2023 census, representing 5.62 per cent of the national total of 241.5 million. This figure is decomposed into 7.41 million males (3.08 per cent of national population) and 6.11 million females (2.54 per cent). Males therefore constitute 54.8 per cent of the 60-plus cohort and females 45.2 per cent, a distribution that differs sharply from what is observed in high-income ageing societies where female longevity advantage typically produces a female-majority elderly population. In Pakistan, as in several other South Asian countries, male numerical dominance persists into old age, reflecting excess female mortality accumulated over the life course from maternal mortality, nutritional disadvantage in childhood and adolescence and lower access to curative healthcare.

The four five-year bands composing the 60-plus population show a clear step-down pattern by age, consistent with normal mortality attrition. The 60-to-64 band is the largest, at 4.99 million total (2.71 million male, 2.28 million female). The 65-to-69 band records 3.57 million (1.92 million male, 1.65 million female). The 70-to-74 band falls to 2.46 million (1.39 million male, 1.08 million female). The 75-and-above open-ended band records 2.50 million (1.39 million male, 1.11 million female), which is marginally larger than the 70-to-74 band. This near-equality between the 70-74 and 75-plus bands is a notable feature: it suggests either that the 75-plus band absorbs persons who would otherwise be in the 70-74 bracket due to age heaping in census reporting, or that survival from 70 to 74 into the 75-plus category is higher than would be expected, possibly because the healthiest individuals selectively survive into the oldest bands. Both interpretations warrant further investigation using vital-statistics data.

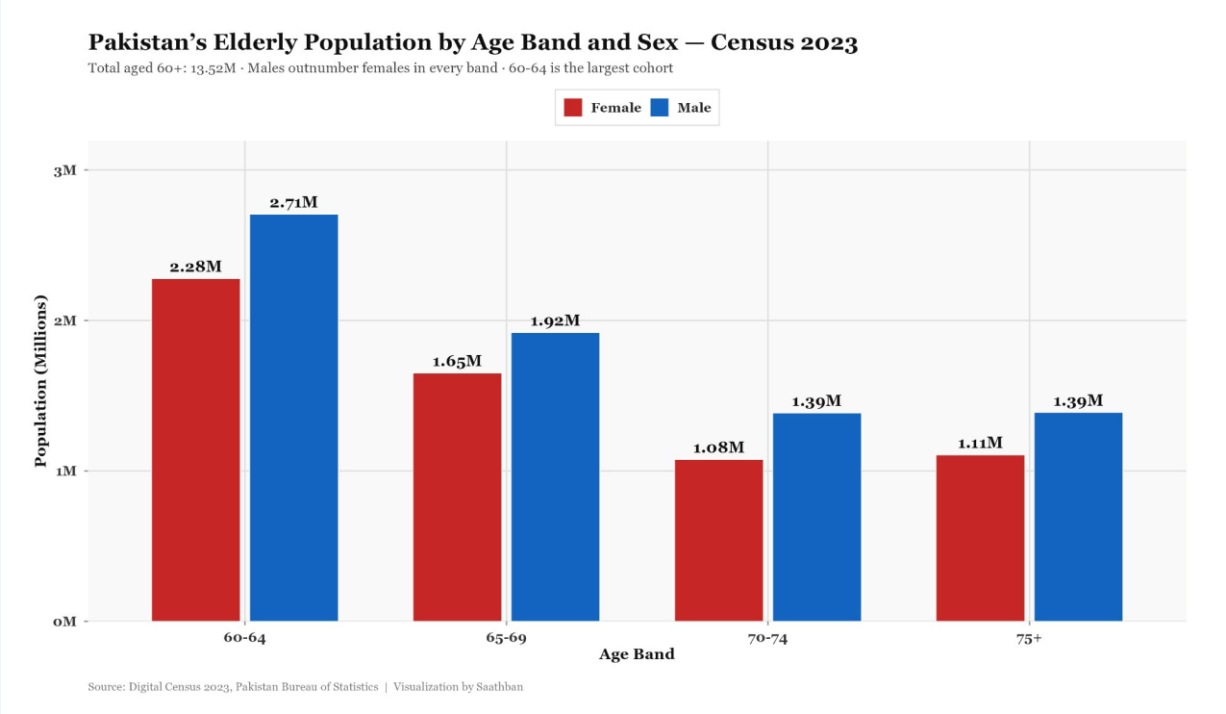

Figure 4.6: Pakistan's Elderly Population by Age Band and Sex: Census 2023

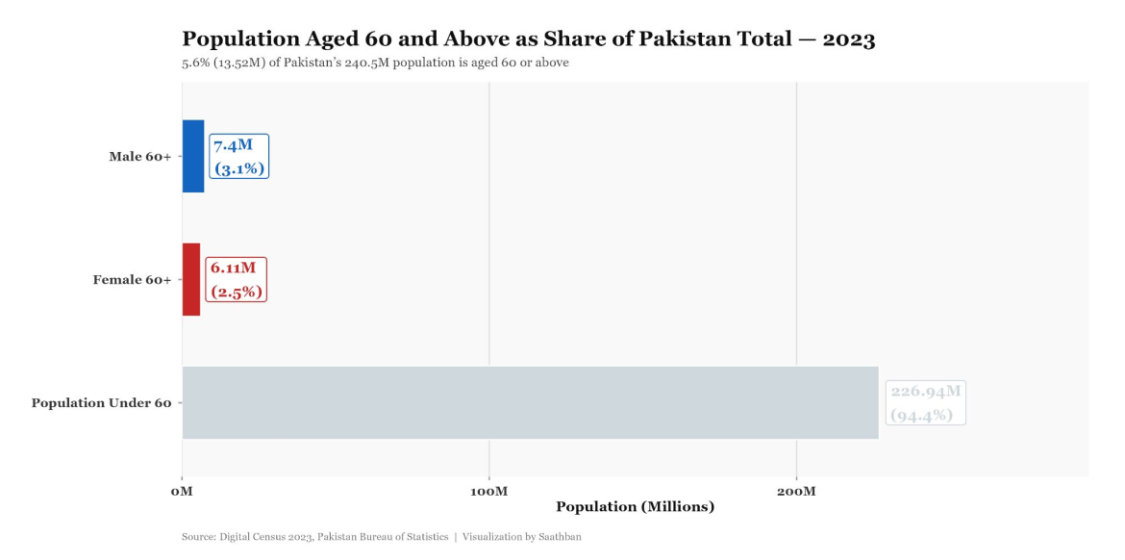

Figure 4.7: Population Aged 60 and Above as Share of Pakistan Total: 2023

4.2.2 The 60-Plus Cohort in National Context

At 5.62 per cent of the national population, Pakistan's 60-plus cohort is small by the standards of South Asian neighbours: India's 60-plus share stood at approximately 10.5 per cent in its most recent full enumeration, and Sri Lanka's exceeds 15 per cent. Bangladesh, which had a near-identical age structure to Pakistan in the 1970s, has now seen its 60-plus share rise to approximately eight per cent following a sharper fertility transition. Pakistan's lower share reflects the combination of a still-high total fertility rate, a young median age of approximately twenty-one years, and the large base of young cohorts documented in sub-section 4.1. However, the absolute number of elderly persons, at 13.52 million, is not small in policy terms. It exceeds the total population of many mid-sized countries and represents a welfare constituency whose needs are structurally underfunded relative to their scale.

Breaking the 60-plus cohort into its constituent bands as a share of the national population makes the graduation of need more concrete. The 60-to-64 band alone accounts for 2.07 per cent of all Pakistanis, equivalent to 4.99 million persons who are approaching or entering retirement, typically experiencing the onset of chronic conditions, and increasingly likely to exit the labour force. The 65-to-69 band at 1.48 per cent (3.57 million) represents the group most acutely in need of regular healthcare access. The 70-to-74 and 75-plus bands, at 1.02 and 1.04 per cent respectively, together number approximately 4.96 million persons aged seventy and above, a group that in the Pakistani context has minimal formal pension coverage, very limited access to long-term care facilities, and is almost entirely dependent on family support structures whose reliability is eroding as urbanisation and nuclear household formation accelerate.

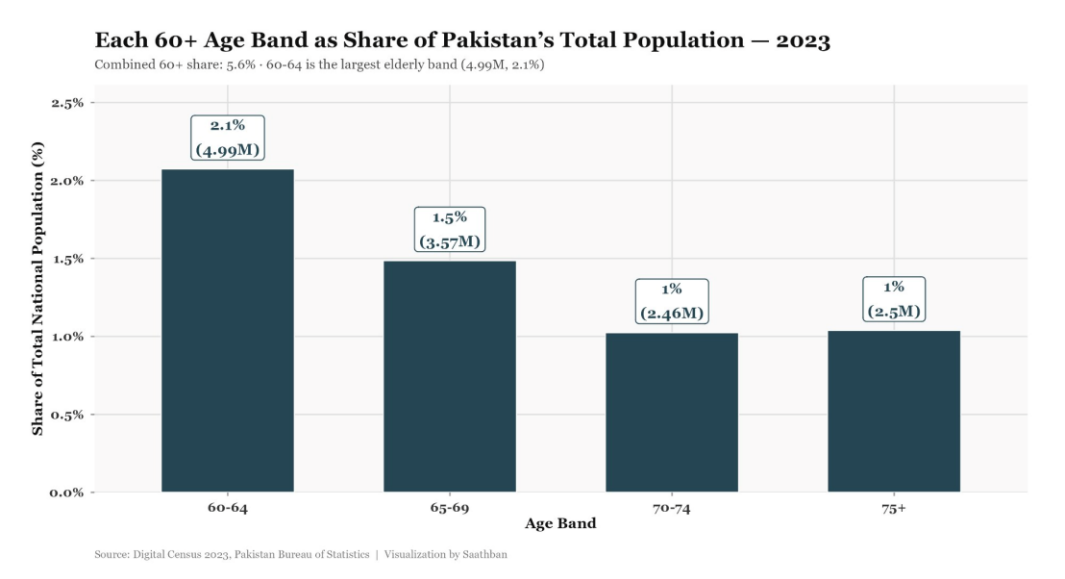

Figure 4.8: Each 60+ Age Band as Share of Pakistan's Total Population: 2023

4.2.3 Sex Ratio Dynamics Within the 60-Plus Cohort

One of the most analytically striking features of Pakistan's elderly population is the behaviour of the sex ratio across age bands within the 60-plus cohort. The sex ratio, expressed as males per 100 females, stands at 118.8 in the 60-to-64 band. It declines slightly to 116.3 in the 65-to-69 band, then rises sharply to 128.7 in the 70-to-74 band before settling at 125.5 in the 75-plus band. The non-monotonic trajectory, with a dip at 65 to 69 followed by a sharp rise, is unusual and merits careful interpretation.

In most populations where data quality is high, the sex ratio falls monotonically with advancing age as female longevity advantage accumulates. The pattern observed in Pakistan, where the ratio rises again at 70 to 74, may reflect several interacting factors. First, age misreporting in census data is well documented in South Asia, with a tendency to report ages in round numbers (70, 75, 80) that systematically redistributes persons across bands. Second, historically high maternal mortality among cohorts now aged 65 to 74 may have removed disproportionately many women from this generation, producing the unusual elevation of the male ratio at older bands. Third, selective migration of elderly males back to ancestral villages for end-of-life care, combined with registration practices that count them in rural origin areas, could distort the apparent ratio depending on how migration is handled in the enumeration. Whatever the mechanism, the persistence of male numerical dominance across all 60-plus bands in Pakistan contrasts sharply with the global pattern and has direct implications for the design of elderly-support programmes, which in many international models are calibrated on the assumption of a female-majority elderly population.

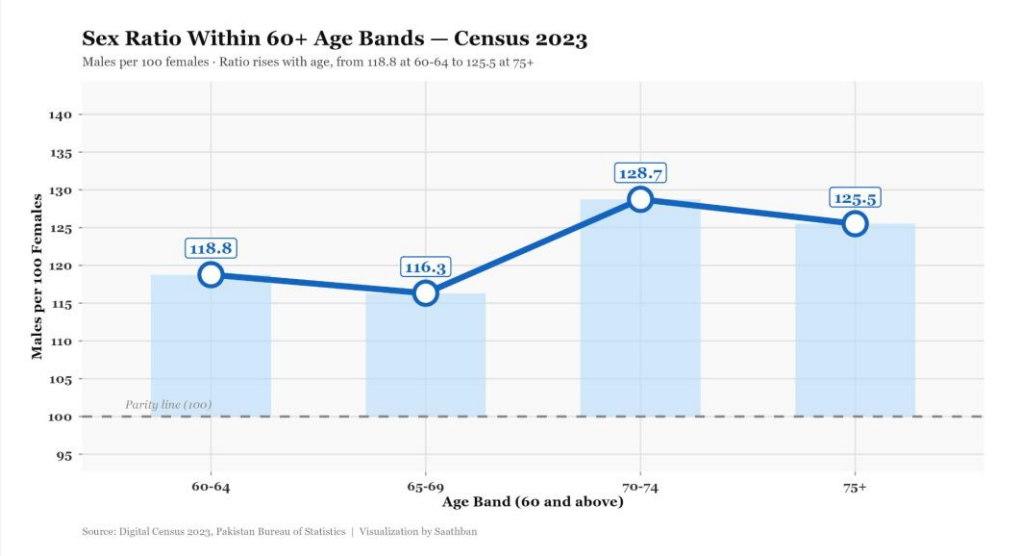

Figure 4.9: Sex Ratio Within 60+ Age Bands: Census 2023

4.2.4 Gender Balance Within Each Elderly Age Band

Examining the internal gender balance within each five-year elderly band, rather than the sex ratio as a single index, adds resolution to the picture. In the 60-to-64 band, males account for 54.3 per cent of all persons in that band and females 45.7 per cent. The male share declines marginally to 53.8 per cent in the 65-to-69 band, consistent with slightly greater male mortality in the early old-age period. It then rises to 56.3 per cent in the 70-to-74 band, consistent with the anomalous sex ratio noted above, before settling at 55.7 per cent in the 75-plus band. The range across the four bands is relatively narrow, between 53.8 and 56.3 per cent male, but the direction of the gradient, rising again after an initial dip, is inconsistent with standard epidemiological expectations and reinforces the hypothesis that data-quality issues or selective mortality by sex are playing a role in the observed distribution.

The policy implication is that Pakistan cannot simply import programme designs calibrated for female-majority elderly populations. If elderly males are numerically dominant and are simultaneously the primary breadwinners in household structures where their incapacitation or death removes the primary income source, then the widowhood effect on female poverty in old age is likely to be severe even if women are a numerical minority. The combination of male numerical dominance and female economic dependence creates a double vulnerability: elderly women are fewer in number but more economically exposed, while elderly men are more numerous but may lack caregiving networks in a society where elder care is conventionally provided by daughters and daughters-in-law.

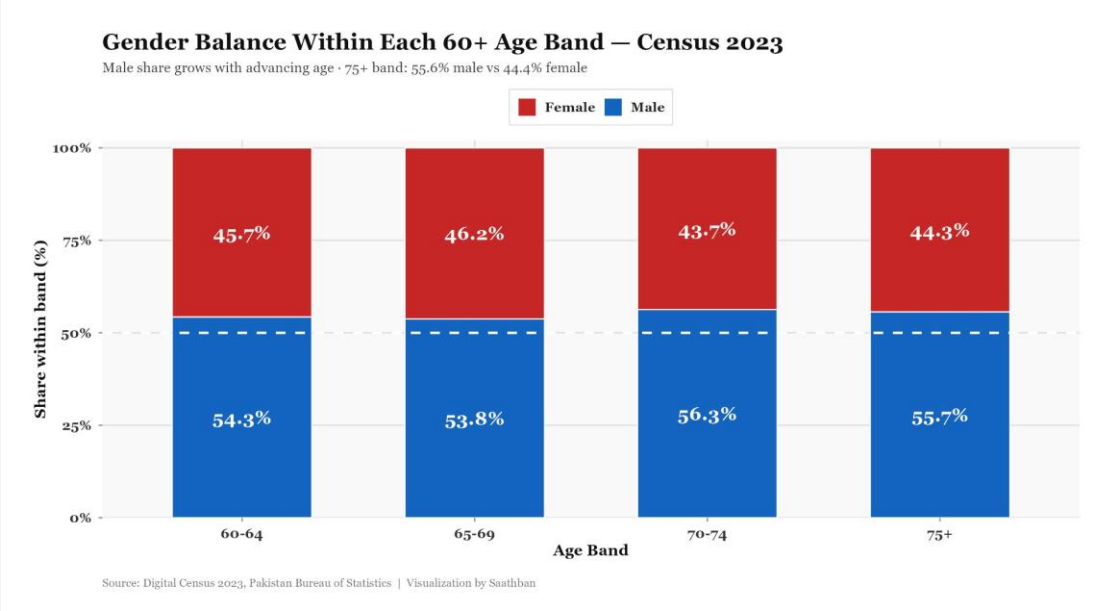

Figure 4.10: Gender Balance Within Each 60+ Age Band: Census 2023

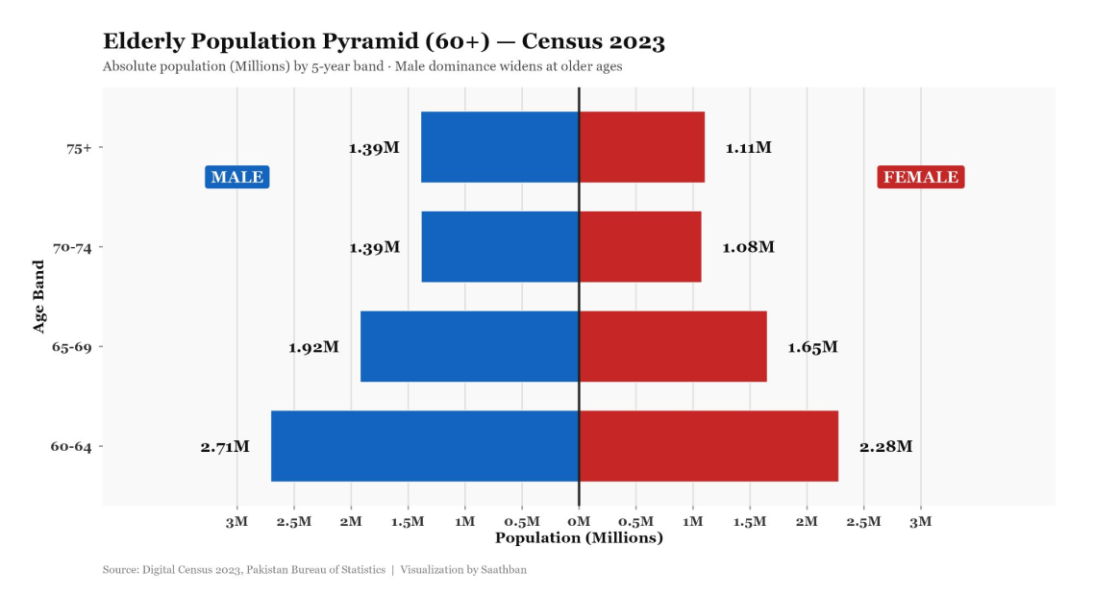

Figure 4.11: Elderly Population Pyramid (60+): Census 2023

4.2.5 Provincial Distribution of the Elderly Population

The provincial distribution of the elderly population, proxied here by the 65-and-above share from the census, reveals meaningful variation that carries distinct policy implications for each province. Islamabad Capital Territory records the highest elderly share at 4.31 per cent of its provincial population, translating to approximately 0.10 million persons aged 65 and above. Despite this being the smallest absolute elderly population of any province, Islamabad's high elderly share relative to its total population reflects the concentration of retired government servants and their dependants, a group with formal pension coverage but increasingly complex healthcare needs as life expectancy extends beyond formal retirement.

Punjab carries the largest absolute elderly population by far, with approximately 5.11 million persons aged 65 and above, representing 4.00 per cent of the province's 127.69 million residents. This is the single largest concentration of elderly persons in any Pakistani province and dwarfs the elderly populations of all other provinces combined. Sindh records 1.70 million elderly persons (3.06 per cent of provincial population), KPK records 1.33 million (3.26 per cent), and Balochistan records just 0.31 million (2.11 per cent). Balochistan's very low elderly share is consistent with its young age structure and reflects both high fertility and persistently elevated adult mortality rates that remove persons from the population before they reach old age. The provincial variation in elderly shares, from 2.11 to 4.31 per cent, also means that a uniform national elderly support policy would be systematically mis-targeted: Balochistan needs more investment in child and maternal health to reduce under-65 mortality, while Punjab and

Islamabad need investment in old-age infrastructure to manage their already-sizeable elderly populations.

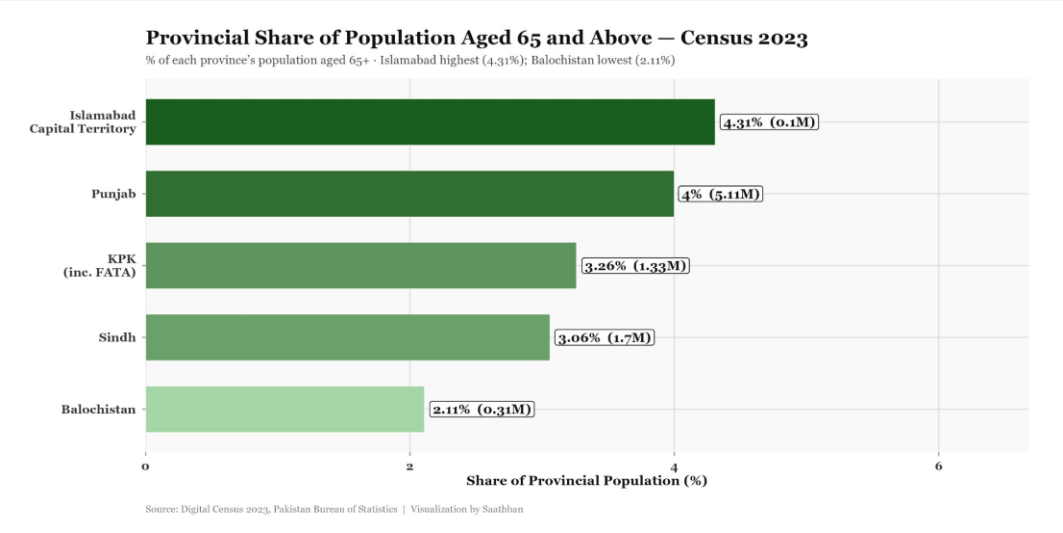

Figure 4.12: Provincial Share of Population Aged 65 and Above: Census 2023

4.3 Section Summary

The Pakistan Census 2023 establishes a clear and empirically grounded picture of a country at an early but active stage of demographic transition. Pakistan's 241.5 million population is growing at 2.55 per cent per year, is dominated by a young age structure in which 41 per cent of persons are below the age of fifteen, and remains a majority-rural society undergoing rapid urbanisation. Provincial heterogeneity is substantial: Balochistan presents a demographic profile closer to sub-Saharan Africa in its youth concentration, while Islamabad begins to exhibit the characteristics of an early-ageing urban society. Gender composition is male-biased at a sex ratio of 106.12, reflecting a combination of data-collection challenges in female enumeration, differential mortality and migration patterns.

Within this predominantly young national structure, the population aged 60 and above has reached 13.52 million persons, or 5.62 per cent of the national total. This cohort is male-dominated in all four five-year bands, with males constituting between 53.8 and 56.3 per cent within each band, a pattern that departs from international norms and reflects historically elevated female mortality in Pakistan. The sex ratio within the cohort follows a non-monotonic trajectory, peaking at 128.7 in the 70-to-74 band, which raises questions about both data quality and the influence of selective mortality across birth cohorts born in the 1940s to 1960s. Punjab holds the largest absolute elderly population at 5.11 million, while Islamabad has the highest

elderly share at 4.31 per cent. Balochistan, with only 0.31 million elderly persons and a 65-plus share of 2.11 per cent, reflects the youngest demographic trajectory of any province.

Taken together, the evidence from this section establishes that Pakistan's elderly population, while still small as a share of the national total, has reached a scale at which it demands dedicated institutional attention. The 13.52 million persons aged sixty and above represent a welfare constituency larger than the total population of many countries in the region. As the large birth cohorts of the 1970s and 1980s approach and enter the 60-plus bracket over the next two to three decades, this cohort will grow with a momentum that cannot be reversed by any policy intervention within a near-term horizon. The analytical groundwork laid in this section provides the demographic context for the policy, fiscal and institutional challenges examined in the remainder of this research.

Source: *Pakistan Bureau of Statistics, Pakistan Digital Census 2023; author's calculations.*

SECTION 5

Laws and Regulations for Elderly Citizens in Pakistan

5.1 Introduction

Pakistan, like many developing nations, is experiencing a gradual but significant demographic shift. The proportion of elderly citizens, defined as individuals aged sixty years and above, is projected to increase substantially in the coming decades owing to declining fertility rates and improvements in life expectancy. This demographic transition presents both an opportunity and a challenge: while older persons represent a repository of knowledge, experience and civic wisdom, they also constitute a population segment that is disproportionately vulnerable to poverty, social exclusion, inadequate healthcare and neglect.

Historically, international human-rights frameworks did not afford specific and enforceable protections to older persons. The situation began to change with the adoption of the United Nations Principles for Older Persons in 1991, followed by the Madrid International Plan of Action on Ageing (MIPAA) in 2002. Pakistan is a signatory to MIPAA, and since its ratification, successive federal and provincial governments have taken steps to enact legislation addressing the welfare, dignity and rights of older citizens.

This section provides a comprehensive review of the legislative and policy framework governing elderly persons in Pakistan, covering international obligations, constitutional provisions, federal enactments and the legislation of all four provinces. Notably, in February 2026, Punjab, the most populous province of Pakistan, enacted the Punjab Senior Citizens Welfare Act 2026, substantially closing the last major provincial legislative gap in this domain.

5.2 International Framework

5.2.1 United Nations Principles for Older Persons (1991)

The United Nations General Assembly adopted the UN Principles for Older Persons in 1991, establishing a foundational international standard for the treatment of elderly populations worldwide. These principles are organised around five core themes: independence, participation, care, self-fulfilment and dignity. They affirm that older persons should have access to adequate food, water, shelter, clothing and healthcare; that they should have the opportunity to remain integrated into society; and that they should be treated fairly and with dignity regardless of age, disability or economic contribution. Although the Principles do not carry binding legal force, they have served as a critical reference point for domestic legislation in Pakistan and across the region.

5.2.2 Madrid International Plan of Action on Ageing (MIPAA, 2002)

The Madrid International Plan of Action on Ageing, adopted at the Second World Assembly on Ageing in April 2002, constitutes the most comprehensive international framework for ageing policy currently in existence. Pakistan is a signatory to MIPAA, which imposes both normative commitments and practical guidance obligations on member states. The Plan identifies three overarching priority directions: the relationship between older persons and development; advancing health and well-being into old age; and ensuring enabling and supportive environments for ageing populations.

MIPAA affirms that population ageing is a global phenomenon of profound consequence, projecting that by 2050 the number of persons aged sixty and above will increase from approximately 600 million to nearly 2 billion, with the most rapid growth occurring in developing countries. It calls on governments to eliminate all forms of discrimination based on age, to guarantee the full enjoyment of all human rights by older persons, and to mainstream ageing into national development frameworks and poverty-eradication strategies. In the Pakistani legislative context, MIPAA has been explicitly cited as a motivating instrument in the preambles of the Islamabad Capital Territory Senior Citizens Act 2021 and several provincial statutes.

The Plan's three priority directions map directly onto Pakistan's legislative response. Provisions in provincial legislation addressing healthcare, geriatric services, financial assistance and old-age homes correspond to MIPAA's health and well-being priority. Provisions addressing housing, transport, public access and legal protection correspond to its enabling-environments priority. Provisions addressing employment, pension facilitation and civic participation correspond to its older-persons-and-development priority.

5.2.3 WHO Active Ageing Framework (2002)

The World Health Organisation's Active Ageing: A Policy Framework, published in 2002, provides a complementary international instrument grounded in the recognition of the human rights of elderly persons and the UN Principles of dignity, economic and social participation, independence and self-fulfilment. The framework emphasises four essential elements: reducing the burden of excess disability, chronic disease and premature mortality; promoting factors that protect health over the lifetime; designing accessible and responsive health and social services; and equipping caregivers with relevant training and education. Pakistan's successive health policies and senior-citizen welfare frameworks have drawn, at least formally, from these international instruments.

5.2.4 Pakistan's International Commitments in Summary

Pakistan's status as a signatory to MIPAA 2002 and its endorsement of the UN Principles for Older Persons together constitute the international legal foundation upon which domestic legislation for elderly citizens has been constructed. The Constitution of the Islamic Republic of Pakistan, 1973, reinforces this framework domestically: Article 37 directs the state to secure the well-being of all citizens irrespective of sex, caste, creed or race, while Article 38 mandates the provision of social security through compulsory social insurance or other means. These constitutional provisions collectively impose an obligation on both the federal and provincial

governments to enact legislation that gives effect to Pakistan's international commitments regarding its elderly population.

5.3 Federal Legislative and Policy Developments

Prior to and following the Eighteenth Constitutional Amendment of 2010, which devolved significant legislative authority to the provinces, the federal government of Pakistan enacted a series of policies, proposals and ordinances relevant to the welfare of elderly citizens. These federal instruments provided the policy architecture upon which provincial legislation was subsequently constructed.

5.3.1 National Policy for Health of Elderly (1999)

The National Policy for Health of Elderly 1999 was Pakistan's first formal policy response to the specific healthcare needs of older persons. Its principal provisions included training and awareness programmes for primary-healthcare physicians in gerontology; ensuring the availability of dental and domiciliary care services; and the development of a multi-tiered healthcare delivery system tailored to the needs of the elderly. Although principally a health-sector document, it established the important precedent that older persons required a specialised, dedicated policy response.

5.3.2 Senior Citizen Relief Package (2004)

In 2004, the Government of Pakistan announced a relief package for senior citizens that included awareness campaigns within educational institutions to cultivate respect for elderly persons among younger generations, and measures to ensure that the basic social and healthcare requirements of senior citizens were addressed through existing government services. While the package lacked the force of formal legislation, it signalled state intent and laid groundwork for subsequent legislative action.

5.3.3 Senior Citizen Bill (2007)

The Senior Citizen Bill 2007, tabled in the National Assembly, proposed a wide-ranging set of measures including concessions on public-transportation fares, facilitation of access to pension and legal services, free entry to public spaces, and the establishment of dedicated service counters for older persons at banks, transport terminals and hospitals. Although the bill was not passed into law at the federal level, its provisions served as a template for subsequent provincial legislation enacted after the Eighteenth Amendment.

5.3.4 ICT Senior Citizens Act (2021)

The Islamabad Capital Territory Senior Citizens Act 2021 (Act No. IX of 2021), passed by the Majlis-e-Shoora and receiving presidential assent on 20 July 2021, provides for the well-being, comfort and dignity of senior citizens residing in the Islamabad Capital Territory. It explicitly

grounds itself in Pakistan's international obligations under the UN Principles for Older Persons and MIPAA 2002.

The Act establishes the ICT Senior Citizens Council under the Ministry of Human Rights as an autonomous body corporate. The Council is composed of the Minister for Human Rights as Chairperson, alongside representatives from civil society, the Chief Commissioner ICT, Finance Division, National Health Services, Pakistan Bait-ul-Maal, Poverty Alleviation Division, the Pakistan Bar Council and members of Parliament from the opposition. Senior citizens on presentation of a Senior Citizen Card are entitled to free entry to public museums, libraries, parks and recreation facilities; express lanes in government establishments; separate medical wards and counters at government hospitals; maximum concession in medical and dental services; 20 per cent discount on medicines and medical fees from designated establishments; 20 per cent concession on domestic travel by airline, railway and government transport; and income-tax exemption up to a prescribed threshold.

The Act further establishes a Dar-ul-Shafqat old-age home for indigent elderly residents of Islamabad and the Senior Citizens Fund. Notably, it includes a maintenance-application mechanism enabling elderly parents unable to maintain themselves to seek financial support from adult children through the Alternative Dispute Resolution framework. A complementary property-protection clause empowers the court to declare void any transfer of assets where the transferee fails to provide the basic necessities of life as agreed. The Grievance Committee established under the Act has the power to impose fines of up to Rs. 50,000 for contraventions, and up to Rs. 1,000,000 in cases of negligence by health-sector entities resulting in loss of life.

5.3.5 Parental Protection Ordinance (2021)

The Parental Protection Ordinance 2021, promulgated by the President of Pakistan through executive authority, directly addresses the growing problem of parents being evicted from their homes by their own children. The Ordinance protects parents from eviction even where the property is legally owned by adult children or has been rented out by them. Elderly parents are also granted the countervailing right to initiate eviction proceedings against children who subject them to abuse or neglect. Law-enforcement authorities are empowered to arrest persons in violation of the Ordinance without a warrant, and convicted offenders are liable to one year of imprisonment, a financial fine, or both. The Ordinance represents a significant step in criminalising domestic abuse directed against older parents and provides swift enforcement mechanisms without requiring elderly victims to navigate complex civil litigation processes.

5.4 Provincial Legislative Frameworks

Following the Eighteenth Amendment to the Constitution (2010), which transferred substantial legislative powers from the federal government to the provinces, each province undertook its own legislative process to enact dedicated senior-citizens legislation. The resulting provincial statutes, while varying in scope and detail, collectively reflect a shared commitment to the welfare, dignity

and rights of older persons. The table below summarises the key features of each provincial Act before they are discussed in turn.

Province	Legislation	Year	Age Threshold	Key Institutions
Khyber Pakhtunkhwa	KPK Senior Citizens Act	2014	60 years	Welfare Council, Welfare Fund
Sindh	Sindh Senior Citizens Welfare Act	2016	60 years	Senior Citizens Council, Welfare Fund, Azadi Card
Balochistan	Balochistan Senior Citizens Act	2017	60 years	Welfare Council, Welfare Fund
Islamabad Capital Territory	ICT Senior Citizens Act	2021	60 years	Council under Ministry of Human Rights, Dar-ul-Shafqat, Fund
Punjab	Punjab Senior Citizens Welfare Act	2026	70 years	Welfare Council (body corporate), Welfare Fund

Table 5.1: Overview of Provincial Senior Citizens Legislation.

5.4.1 Khyber Pakhtunkhwa Senior Citizens Act (2014)

Khyber Pakhtunkhwa became the first province to enact comprehensive senior-citizens legislation through the KPK Senior Citizens Act 2014. The Act established a Senior Citizen Welfare Council and a dedicated Senior Citizen Welfare Fund. It mandated the conduct of relevant research to inform ongoing policy development and prescribed the provision of health services, financial support and housing facilities for eligible elderly citizens aged sixty years and above. The KPK Act set an important precedent that was subsequently followed by the other provinces, though the detail and scope of later legislation progressively expanded upon this foundation.

5.4.2 Sindh Senior Citizens Welfare Act (2016)

The Sindh Senior Citizens Welfare Act 2016 (Sindh Act No. XXI of 2016), passed by the Provincial Assembly of Sindh on 29 April 2016 and assented to by the Governor on 21 June 2016, is among the most detailed provincial senior-citizens statutes. It defines a senior citizen as any permanent resident of Sindh aged sixty years or above and establishes the Senior Citizens Council as a body corporate headquartered at Karachi, with district-level offices across the province.

The Council's functions are wide-ranging. It is mandated to formulate welfare policies; issue the Senior Citizens Card (known as the Azadi Card), which confers access to a comprehensive package of benefits; establish lodging establishments and old-age homes for the physical, mental, emotional, social and economic well-being of senior citizens; and promote civic participation by leveraging the knowledge, skills and experience of older persons. The Act provides for a 50 per cent concession on road-transport fares and a 25 per cent discount on the purchase of goods,

medicines and essential commodities. Senior citizens are also entitled to a 25 per cent discount at recreation centres, cinemas, theatres, hotels, motels, resorts and restaurants, as well as free geriatric, medical and health services at government facilities and a 25 per cent concession at private hospitals, medical centres and clinics.

The Act contains notable enforcement provisions: any person who refuses to honour the senior-citizen card or violates its provisions is liable to rigorous imprisonment of not less than one year but not more than three years, or a fine of up to Rs. 30,000, or both. A separate provision criminalises the exposure and abandonment of a senior citizen by a family member or spouse, punishable with imprisonment of three months or a fine. The Act also establishes the Senior Citizens Welfare Fund, audited annually by the Auditor General of Pakistan, and includes a significant property-protection clause providing that any transfer of property conditioned on the care of the transferor is void where the transferee fails to provide that care.

5.4.3 Balochistan Senior Citizens Act (2017)

The Balochistan Senior Citizens Act 2017 (Act No. III of 2017), passed by the Provincial Assembly of Balochistan on 20 May 2017 and assented to by the Governor on 7 June 2017, applies to the whole of Balochistan excluding Tribal Areas. It establishes the Balochistan Senior Citizens Welfare Council as an independent, non-profit body corporate headquartered at Quetta. The Council is chaired by an eminent senior citizen of good standing nominated by the Chief Minister, and its membership includes secretaries from Finance, Planning and Development, Health, Home and Law Departments, all divisional commissioners, the Director General Social Welfare, Director Bait-ul-Maal, and representatives of non-governmental organisations.

The Act provides senior citizens aged sixty and above with a Senior Citizen Card upon which they are entitled to free entry to public museums, libraries, parks and recreation facilities; dedicated counters at hospitals, public offices, banks and railway stations; concessions on medical and medicine charges; and separate medical wards. Financial support is extended to families responsible for the care of their elderly relatives. The Act mandates the establishment of senior-citizen homes for homeless, deserted and indigent aged persons, and promotes the inclusion of geriatrics in medical-university curricula. The Balochistan Senior Citizens Welfare Fund is established as the primary financing vehicle, funded by government allocations, private donations and contributions from Pakistan Bait-ul-Maal.

5.4.4 Punjab Senior Citizens Welfare Act (2026)

The Punjab Senior Citizens Welfare Act 2026 (Act XI of 2026), passed by the Provincial Assembly of the Punjab on 4 February 2026 and assented to by the Governor on 25 February 2026, resolves the longstanding legislative gap in Punjab's elderly-protection framework. For over a decade, Punjab, Pakistan's most populous province with over 110 million residents, remained the only major province without an enacted senior-citizen welfare statute. The Punjab Senior Citizens Welfare and Rehabilitation Bill 2013 had been drafted but never passed, a gap that the 2026 Act now closes.

Age Threshold and Definitions

The Act defines a senior citizen as an elderly person who has attained seventy years of age or more and is residing in Punjab. This threshold is notably higher than the sixty-year standard used in all other provincial statutes and in international frameworks such as MIPAA, and its implications for coverage are significant. The Act separately defines the category of indigent senior citizens, meaning those who are frail, sickly or living with a disability and who lack a pension, permanent income or financial support from relatives adequate to cover their basic needs. Welfare is defined broadly to include all services relating to social, economic, boarding, lodging and legal-protection needs of senior citizens.

Punjab Senior Citizens Welfare Council

The Act establishes the Punjab Senior Citizens Welfare Council as a body corporate with perpetual succession and a common seal. The Council is headquartered at Lahore and may establish sub-offices on a needs basis anywhere in Punjab. Its composition is as follows:

Designation	Role
Minister for Social Welfare	Chairperson
Secretary, Social Welfare and Bait-ul-Maal Department	Vice Chairperson / Member
Secretary, Specialised Healthcare and Medical Education Department (or nominee)	Member
Secretary, Primary and Secondary Healthcare Department (or nominee)	Member
Secretary, Home Department (or nominee)	Member
Secretary, Finance Department (or nominee)	Member
Secretary, Local Government and Community Development Department (or nominee)	Member
Secretary, Housing, Urban Development and Public Health Engineering Department (or nominee)	Member
Secretary, Planning and Development Department (or nominee)	Member
Chief Executive Officer, Punjab Social Protection Authority	Member
Three members of Provincial Assembly including one female member	Members
Four philanthropists including two women from NGOs and civil society	Members
Director General, Social Welfare and Bait-ul-Maal Department	Member / Secretary

Table 5.2: Composition of the Punjab Senior Citizens Welfare Council.

The Chairperson and all Members serve pro bono. The Council holds at least quarterly meetings, with a quorum of one-third of total membership and decisions taken by majority vote, with the presiding officer holding a casting vote in the event of equality.

Powers and Functions

The Council's statutory mandate is comprehensive. It is required to conduct research and compile data on ageing; formulate and submit policy proposals in light of national and international commitments; arrange workshops and public-awareness campaigns; undertake welfare activities in collaboration with public and private sectors; mobilise financial resources; and coordinate with governmental, non-governmental and international organisations. The Council is further mandated to establish old-age homes for homeless, deserted and indigent senior citizens; coordinate with government hospitals for free medical treatment and geriatric care; coordinate with medical colleges for the introduction of geriatrics as a subject in their curricula; coordinate with bar councils for the provision of free legal aid to deserving senior citizens; coordinate for separate counters, transport facilitation and ramps in public areas; promote the rights of senior citizens across social, print and electronic media; and establish a toll-free helpline for guidance, assistance and grievance handling.

Punjab Senior Citizens Welfare Fund

The Act establishes the Punjab Senior Citizens Welfare Fund, which vests in the Council and is to be utilised for the performance of its functions as prescribed by regulations. The Fund consists of government funds and grants; grants from other authorities and agencies; domestic and national donations; international donations subject to government approval; and all other sums received by the Council. Accounts are to be maintained at scheduled banks as prescribed, and audited annually by the Auditor-General of Pakistan. The Council may also engage a registered chartered-accountant firm for supplementary audit. Within four months of the end of each financial year, the Council is required to submit an annual performance report to the Provincial Assembly of the Punjab, including statements of accounts, audit reports, activities, targets achieved and future plans.

Administrative and Governance Framework

The Secretary of the Council also serves as Director General and administrative head of the Council, responsible for all principal financial and administrative functions including maintenance of records and the common seal, authorising payments, preparing budget estimates, overseeing procurements and executing deeds on behalf of the Council. The Director General may delegate powers with the Council's approval and serves pro bono. Officers and employees appointed by the Council are not civil servants under the Punjab Civil Servants Act 1974, but are deemed public servants under section 21 of the Pakistan Penal Code 1860. The Government retains the power to issue binding directions to the Council and to transfer existing government

employees of the Social Welfare and Bait-ul-Maal Department to the Council on terms no less favourable than those previously applicable.

5.5 Consolidated Overview of the Legislative Framework

The following tables provide a comprehensive summary of Pakistan's federal and provincial legislative landscape for elderly citizens and a comparative analysis of key provisions across all provincial statutes.

Legislation / Policy	Year	Level	Key Focus Areas
National Policy for Health of Elderly	1999	Federal	Healthcare delivery, gerontology training, domiciliary and dental care
Senior Citizen Relief Package	2004	Federal	Awareness campaigns, provision of basic social and health needs
Senior Citizen Bill	2007	Federal (not enacted)	Transport concessions, priority counters, free access to public spaces
KPK Senior Citizens Act	2014	KPK Province	Welfare Council, Welfare Fund, health services, housing, financial support
Sindh Senior Citizens Welfare Act	2016	Sindh Province	Welfare Council, Azadi Card, welfare homes, 50% transport concession, criminal sanctions
Balochistan Senior Citizens Act	2017	Balochistan Province	Welfare Council, Welfare Fund, old-age homes, family financial support, free public access
ICT Senior Citizens Act	2021	ICT / Federal	Dar-ul-Shafqat, Senior Citizens Fund, 20% concessions, maintenance applications, property protection
Parental Protection Ordinance	2021	Federal (Presidential)	Anti-eviction protection for parents, warrantless arrest, one-year imprisonment penalty
Punjab Senior Citizens Welfare Act	2026	Punjab Province	Welfare Council (body corporate), Welfare Fund, old-age homes, free legal aid, toll-free helpline

Table 5.3: Federal and Provincial Legislative Framework for Elderly Citizens in Pakistan.

Provision	KPK (2014)	Sindh (2016)	Balochistan (2017)	ICT (2021)	Punjab (2026)
Age Threshold	60 years	60 years	60 years	60 years	70 years
Welfare Council	Yes	Yes	Yes	Yes (MoHR)	Yes (body corporate)
Welfare Fund	Yes	Yes	Yes	Yes	Yes

Provision	KPK (2014)	Sindh (2016)	Balochistan (2017)	ICT (2021)	Punjab (2026)
Old-Age Homes	Not specified	Yes	Yes	Yes (Dar-ul-Shafqat)	Yes
Financial Assistance	Yes	Via NGOs	Yes (to families)	Yes (indigents)	Yes (indigents)
Transport Concession	Not specified	50% on road transport	Not specified	20% on domestic travel	Coordination only
Medicine Concession	Not specified	25% + free at govt hospitals	Concession in charges	20% from designated outlets	Free at govt hospitals
Free Public Access	Not specified	Not specified	Museums, libraries, parks	Parks, museums, libraries	Museums, libraries, parks
Separate Counters	Not specified	Not specified	Yes	Yes	Yes (coordination)
Legal Aid	Not specified	Not specified	Not specified	Not specified	Yes (bar councils)
Geriatric Education	Not specified	Not specified	Yes (medical colleges)	Not specified	Yes (medical colleges)
Toll-Free Helpline	No	No	No	No	Yes
Maintenance Clause	Not specified	Not specified	Not specified	Yes	Not specified
Property Protection	Not specified	Yes (void transfer)	Not specified	Yes (void transfer)	Not specified
Criminal Sanction	Not specified	Yes (1–3 yrs or fine)	Not specified	Yes (fines)	Not specified

Table 5.4: Comparative Analysis of Provincial Senior Citizens Legislation.

5.6 Critical Analysis: Gaps and Implementation Challenges

Despite the commendable volume of legislative activity at both the federal and provincial levels, significant challenges persist in the practical implementation and enforcement of elderly-rights legislation in Pakistan. Several systemic deficiencies have been identified in the literature and in policy analyses.

The most immediate concern is definitional inconsistency. The Punjab Senior Citizens Welfare Act 2026 defines a senior citizen as a person aged seventy years or more, while all other provincial statutes and international frameworks use sixty years. This inconsistency creates an anomalous

situation in which residents of Pakistan's most populous province must be ten years older than their counterparts elsewhere to qualify for equivalent statutory protection. Given that average life expectancy in Pakistan hovers around 67 to 68 years, the 70-year threshold may effectively exclude a significant portion of the elderly population from the Act's coverage. Harmonisation of the age threshold with the sixty-year standard used in MIPAA and in other provincial legislation is a priority reform.

Enforcement deficits remain a pervasive challenge. Most provincial statutes, including the Punjab 2026 Act, lack robust enforcement mechanisms, dedicated inspectorates or designated judicial forums for the resolution of elderly-rights violations. The Sindh Act is the exception in providing criminal sanctions of up to three years' imprisonment for refusal to honour senior-citizen card privileges, but even there, enforcement in practice has been limited. The existence of legislation on paper does not, without effective enforcement, translate into meaningful protection for vulnerable older persons.

Resource allocation is a related concern. The establishment of welfare funds, residential homes and subsidised services as envisaged in provincial legislation has not been uniformly accompanied by corresponding budgetary allocations, limiting operational effectiveness. The Punjab Act's Fund provisions must be accompanied by meaningful appropriations to be effective. Similarly, the Balochistan and KPK Acts established funds but have faced challenges in capitalising them at levels commensurate with the scale of need.

The participation of elderly persons in the design and review of legislation that directly affects them has also been limited, contrary to the participatory principles envisaged in MIPAA and other international frameworks. Meaningful consultation with older citizens and their representative organisations in the legislative and policy process would strengthen both the substance and the legitimacy of elderly-protection frameworks.

Finally, while the Punjab 2026 Act comes into force immediately upon assent, the establishment of the Council, appointment of members, operationalisation of the Fund, and roll-out of services such as the toll-free helpline and old-age homes will require sustained governmental commitment over time. Early and systematic monitoring of implementation progress will be essential to ensure that the legislative architecture translates into tangible improvements in the lives of elderly Punjabis.

5.7 Conclusion

Pakistan's legal framework for the protection of elderly citizens has evolved substantially over the past two and a half decades, progressing from general constitutional obligations and health-sector policies in the late 1990s to specific, enforceable statutory protections at both the federal and provincial levels. The international architecture of MIPAA 2002, the UN Principles for Older Persons, and the WHO Active Ageing Framework has provided the normative foundation upon which this domestic legislative response has been built.

At the federal level, the Parental Protection Ordinance 2021 represents a significant step in criminalising domestic abuse against older parents, while the ICT Senior Citizens Act 2021 provides a comprehensive model for the federal capital. At the provincial level, the enactment of the Punjab Senior Citizens Welfare Act 2026 closes the last major provincial legislative gap, bringing Pakistan's most populous province within the ambit of a formal statutory framework for senior-citizen welfare. Together with the KPK Act of 2014, the Sindh Act of 2016, and the Balochistan Act of 2017, Pakistan now has a national mosaic of elderly-protection legislation covering all major territories.

Nevertheless, the overall framework remains incomplete and unevenly implemented. The Punjab Act's 70-year age threshold, the absence of robust enforcement mechanisms across most provincial statutes, limited budgetary commitments, and the constrained participation of elderly persons in policymaking processes represent critical gaps requiring ongoing attention. The effectiveness of Pakistan's commitment to elderly rights will ultimately be measured not by the volume of legislation enacted, but by the degree to which that legislation meaningfully improves the lived experience of older citizens across all provinces and territories of the country.

SECTION 6

Elderly Care Services in Pakistan

Who serves Pakistan's elderly, how they serve, and whom they reach

6.1 Introduction

Formal elderly care in Pakistan is carried out by a modest but varied set of organisations. Care for older people has traditionally been a family responsibility, and the formal sector is therefore comparatively young. It has, however, grown into a recognisable ecosystem made up of government welfare departments, well-established charitable foundations, smaller faith-based institutions, specialised medical bodies and a newer generation of private residential and home-based services.

This section describes that ecosystem. Rather than listing organisations one by one, it groups them according to the sector they belong to and the kind of service they deliver. For each organisation, the section gives the founder or current lead and the role that person plays, so that the human face of these services is visible alongside the institutional description. A complete directory of the nineteen organisations covered in this study, with the cities they serve and the beneficiaries they target, is provided as an appendix at the end of the section.

6.2 The Public Sector

Government provision focuses on the destitute and those without family support rather than on older citizens as a whole. Three distinct streams operate within this sector.

Punjab Social Welfare Department, Old Age Homes (Aafiat)

The Punjab Social Welfare Department runs seven old-age homes across Lahore, Multan, Rawalpindi, Faisalabad, Narowal, Sahiwal and Toba Tek Singh. The first home was set up in Lahore in 1975 under the name Aafiat, and the seven homes together have a capacity of 300 residents, at fifty per home. Admission is free for Punjab residents aged sixty and above, without distinction of gender, caste, creed or religion, and services cover shelter, food, healthcare and recreational facilities. The department is a line department of the Government of Punjab and is headed by the Secretary, Social Welfare and Bait-ul-Maal, Punjab.

Pakistan Bait-ul-Maal

Pakistan Bait-ul-Maal is an autonomous federal body working under the Prime Minister's Secretariat. It was created under the Pakistan Bait-ul-Maal Act of 1991 and began operations in 1992. Its mandate covers the destitute, widows, orphans, the disabled and the elderly. The organisation operates old-age homes in Islamabad, Lahore, Karachi, Peshawar and Quetta, each

with around fifty places, and provides food, shelter, clothing, healthcare and recreational activities. Its day-to-day affairs are run by a Managing Director appointed by the federal government.

Ehsaas Panagah Programme

The Ehsaas Panagah Programme was launched under the federal social-protection umbrella and extended through the big cities of Pakistan. Panagahs provide overnight accommodation and free meals at several sites in Lahore as well as in Multan, Faisalabad, Sargodha, Rawalpindi, Quetta, Karachi and Peshawar. Although not designed exclusively for older people, elderly citizens without shelter form a significant share of Panagah beneficiaries. The programme has now been either integrated into Bait-ul-Maal services or discontinued altogether in certain provinces.

6.3 Large Charitable Foundations

The charitable sector carries the largest share of formal elderly care in the country, both in the number of residents housed and in geographical reach. Five well-established foundations lead this tier.

Edhi Foundation

The Edhi Foundation was founded in 1951 in Karachi by Abdul Sattar Edhi, the humanitarian widely recognised as the country's most prominent social worker, who led the organisation until his death in 2016. His wife Bilquis Edhi, a trained nurse, ran the maternity, cradle and adoption services until her passing in 2022. The organisation is now led by Faisal Edhi, Chairman, who took over in 2016 and continues to run operations from the Karachi headquarters. Kubra Edhi serves as Director. The foundation operates old-age homes in Karachi, Multan, Lahore, Islamabad, Peshawar, Quetta and Chitral, offering free shelter, food, medical care and burial services to abandoned and destitute elderly.

Alkhidmat Foundation Pakistan

Alkhidmat Foundation Pakistan was registered as a non-governmental organisation in 1990 and is the welfare wing of Jamaat-e-Islami Pakistan. It is headquartered in Lahore and runs one of the largest elderly-care networks in the country, with twenty-four homes spread across major and smaller cities. The current President is Prof. Dr. Hafeez-ur-Rahman, and the Secretary General is Syed Waqas Jafri. Dr. Mushtaq Ahmed Mangat serves as Vice President. The foundation works through a national network of paid staff and tens of thousands of volunteers and covers disaster management, healthcare, education and orphan care alongside its elderly homes.

Chhipa Welfare Association

Chhipa Welfare Association was founded in 1987 in Karachi by Muhammad Ramzan Chhipa, a social worker who established the organisation after the Bohri Bazaar bombings in that year. Mr. Chhipa continues to lead the organisation as its Patron-in-Chief. Chhipa Old Home, located on

Shahrah-e-Faisal in Karachi, provides shelter, meals, and health and hygiene care to senior citizens without discrimination of creed, colour or caste. It is run on donations and forms part of a broader family of Chhipa services that includes ambulances, a newborn home, orphanage, women's shelter and free kitchen.

JDC Foundation Pakistan

The JDC Foundation, short for Jafaria Disaster Management Cell Welfare Organization, was established in 2009 in Karachi by Syed Sibt-e-Jaffar Zaidi together with a group of younger volunteers including Syed Zafar Abbas Jafri, who serves as General Secretary and is the organisation's public face. JDC operates old-age homes in Karachi, the best known of which is Gulistan-e-Zainabia in Abbas Town, offering twenty-four-hour care, meals, hygiene support and emotional support to elderly men and women without family to rely on.

Bahria Town Old Age Home

The Bahria Town Old Age Home in Lahore was established in June 2014 under the Begum Akhter Rukhsana Memorial Trust of Bahria Town, the private real-estate group led by its Chairman Malik Riaz Hussain. The home has a capacity of around thirty-five to forty residents and provides free shelter, food, medical treatment and rehabilitation to homeless and destitute elderly, with the stated aim of offering a home-like atmosphere with dignity and care.

6.4 Smaller and Faith-Based Homes

Dar-ul-Sukoon

Dar-ul-Sukoon was founded on 17 February 1969 by Sister Gertrude Lemmens, a Dutch Franciscan nun who had arrived in Karachi in 1939. She led the organisation until her death in October 2000. Her successor, Sister Ruth Lewis of the Franciscan Missionaries of Christ the King, continued the work and oversaw the expansion of the centres until her death in July 2020. Dar-ul-Sukoon has around 150 residents at its main centre on Kashmir Road and operates a network of centres in Karachi that includes Peace Haven for older and destitute residents. Although originally set up for children with disabilities, the organisation today also provides residential care to the elderly. It is governed by a board drawn from religious, social and business institutions.

Anmol Zindagi Old Age Home

Anmol Zindagi was founded in January 2011 in North Nazimabad, Karachi, by Amanat Parwaiz, a physiotherapist who had retired from Sindh Medical College and began the home after caring for an elderly patient through home visits. He serves as Founder and Chairman and, with his family, lives on the premises. The home distinguishes itself by addressing both physical and mental well-being and offers twenty-four-hour nursing, doctor visits, physiotherapy, yoga and mental-health support.

Aga Khan Foundation Senior Citizen Home, Karimabad

The Senior Citizen Home at Karimabad, North Nazimabad, in Karachi operates under the Aga Khan Development Network and provides residential care and community support reserved exclusively for members of the Ismaili community. It is governed within the council structures of the Ismaili community in Pakistan and is not open to non-members.

6.5 Advocacy and Specialist Medical Bodies

Pakistan Senior Citizens Association (PSCA)

The Pakistan Senior Citizens Association was founded in 1985 and is based in Clifton, Karachi. It is one of the earliest senior-citizen bodies in the country and combines advocacy work with direct support such as mobility equipment, wheelchairs, community events and assistance to destitute older people. It operates as a membership body serving older citizens across income levels and is run by an elected office-bearer structure, with the Office Secretary handling day-to-day coordination.

Geriatric Care Foundation Karachi (GCFK)

The Geriatric Care Foundation Karachi, based on Arambagh Road, is one of the few dedicated geriatric medical bodies in Pakistan. It concentrates on healthcare for medically vulnerable older adults and works through a board of medical professionals. Its stated aim is to bring clinical attention specifically to the diseases and conditions of old age, which are otherwise rarely the focus of general hospitals in the country.

6.6 Private and Paid Care

A smaller but growing segment of the ecosystem caters to families able to pay for continuity of care. This segment includes one private residential facility and several home-based services.

The Second Home (formerly Fraternity Old Age Home)

The Second Home in Lahore, run by Holistic Healthcare Services, has been operating since 2006 and describes itself as the first certified elderly-care facility in Pakistan. It reports having served around 5,000 patients. It offers assisted living, dementia and Alzheimer's care, physiotherapy, speech therapy, palliative care and stroke care.

PlanCare

PlanCare is a Lahore-based home-care provider also operating in other major cities. It offers home nursing, caregiver services, medication management, dementia care, post-acute rehabilitation and hygiene support, and markets itself both to nuclear families in Pakistan and to overseas Pakistanis caring for parents from abroad. The service operates on structured care plans drawn up after a professional assessment of each elderly client.

PrivaCare

PrivaCare, based in Lahore, describes itself as a leading home-based senior-care provider and uses an Interactive Caregiving model that aims to engage older clients physically, mentally and socially. Its services include in-home elder care, respite, companion care, mobility assistance, medication management, physiotherapy and lab home visits.

Pakheal

Pakheal is a Lahore-based service offering round-the-clock home elderly care that includes home nursing, post-surgical care, palliative and hospice support, chronic-illness management, mobility assistance, companionship and incontinence care. It positions itself as a cost-effective alternative to nursing homes, allowing older people to remain in familiar home environments.

Elders Club / Elders Care Pakistan

Elders Club was established in 2022 and operates across multiple cities in Pakistan through an online enrolment model. It reports support to more than 43,000 senior citizens and offers wellness programmes, health monitoring, medication management, personal assistance, social engagement and home care. Family members are encouraged to enrol their elders on their behalf, making the platform particularly relevant for children living away from their parents.

6.7 Hospital-Based Care

Senior Citizens Primary Care Unit (SCPCU), Liaquat National Hospital, Karachi

The Senior Citizens Primary Care Unit at Liaquat National Hospital in Karachi was inaugurated in October 2019 and is described by the hospital as the first of its kind within Pakistani healthcare. The unit is purpose-built for older patients, with handrails, non-slip flooring and alarms, and brings together physician visits, nursing, psychology, physiotherapy, occupational therapy, a dietitian, grooming and mobility support in a single daycare facility. It is operated by Liaquat National Hospital and Medical College under the leadership of its Chief Executive Officer and clinical department heads.

6.8 Reading the Ecosystem as a Whole

Taken together, the organisations above show that elderly care in Pakistan is delivered by four kinds of actor. The state provides a safety net through the Punjab Social Welfare Department, Pakistan Bait-ul-Maal and the Ehsaas Panagah programme. Large charitable foundations carry the bulk of operational work, with Edhi, Alkhidmat, Chhipa, JDC and the Bahria Town home reaching the destitute across the country. Smaller and specialist organisations such as Dar-ul-Sukoon, Anmol Zindagi, the Aga Khan home, the Pakistan Senior Citizens Association and the Geriatric Care Foundation Karachi fill specific niches in Karachi. A newer private tier, centred in Lahore, is building residential and home-based services for families that can pay, while Liaquat

National Hospital provides the only dedicated hospital-based geriatric daycare unit recorded in this directory.

The geographical picture is striking. Karachi and Lahore hold most of the provision, followed by Islamabad, Rawalpindi, Multan, Faisalabad, Peshawar and Quetta, which appear largely through the presence of nationwide networks. Outside these corridors, formal elderly care is thinly documented. The directory at the end of this section allows the reader to see the full distribution in one place and serves as a reference for the policy discussion that follows in later chapters of the Saathban research programme.

Appendix 6A: Directory of Organisations Reviewed

The following table lists the nineteen organisations covered in this section, with sector, principal cities of operation, and the main beneficiary group served by each.

#	Organisation	Sector	City / Reach	Main Beneficiaries
1	Punjab Social Welfare Department, Old Age Homes (Aafiat)	Government	Lahore, Multan, Rawalpindi, Faisalabad, Narowal, Sahiwal, Toba Tek Singh	Destitute and low-income elderly aged 60+
2	Pakistan Bait-ul-Maal	Government, autonomous	Islamabad, Lahore, Karachi, Peshawar, Quetta	Destitute and vulnerable elderly, widows, disabled
3	Ehsaas Panagah Programme	Government	Lahore, Multan, Faisalabad, Sargodha, Rawalpindi, Peshawar, Quetta	Marginalised and homeless, including elderly
4	Edhi Foundation, Old Age Homes	Large charity	Karachi, Multan, Lahore, Islamabad, Peshawar, Quetta, Chitral	Abandoned and destitute elderly
5	Alkhidmat Foundation Pakistan, Old Age Homes	Large charity	Nationwide	Destitute elderly without family support
6	Chhipa Welfare Association, Old Home	Large charity	Karachi	Destitute elderly of all backgrounds
7	JDC Welfare Organization, Old Age Homes	Charity	Karachi	Elderly without family support
8	Dar-ul-Sukoon	Faith-based NGO	Karachi	Elderly and disabled

#	Organisation	Sector	City / Reach	Main Beneficiaries
9	Anmol Zindagi Old Age Home	NGO	Karachi, North Nazimabad	Senior citizens aged 60+
10	Pakistan Senior Citizens Association	Advocacy NGO	Karachi	Senior citizens across income levels
11	Geriatric Care Foundation Karachi	Medical NGO	Karachi	Medically vulnerable elderly
12	The Second Home (formerly Fraternity Old Age Home)	Private residential	Lahore	Paying middle and upper-class elderly
13	Bahria Town Old Age Home	Corporate charity	Lahore	Homeless and destitute elderly
14	PlanCare	Private home-based	Lahore and other major cities	Home-based elderly across conditions
15	PrivaCare	Private home-based	Lahore	Home-based elderly
16	Pakheal	Private home-based	Lahore	Home-based elderly
17	Elders Club / Elders Care Pakistan	Private home-based platform	Multiple cities	Senior citizens aged 60+
18	Liaquat National Hospital, Senior Citizens Primary Care Unit	Hospital-based	Karachi	Senior citizens needing medical daycare
19	Aga Khan Foundation, Senior Citizen Home, Karimabad	Community faith-based	Karachi	Ismaili-community elderly

Appendix Table 6A.1: Directory of Nineteen Organisations Providing Elderly-Care Services in Pakistan.

SECTION 7

What Comes Next

Two additional sections are in draft preparation and will be issued in the revised edition of this report. They are listed here so that the reader has a full view of the intended scope of the Saathban Secondary Report.

7.1 Lived Experience of Older Pakistanis

A dedicated treatment of the economic, health-related, social and psychological challenges faced by the elderly in Pakistan, drawing on peer-reviewed literature, NGO field reports and government surveys. This section will set the qualitative context against which the demographic and institutional findings of the earlier sections should be read.

7.2 Recommendations and Policy Directions for Saathban

A forward-looking set of policy recommendations, grouped by time horizon and by the level of government responsible for implementation. Drawing on the evidence compiled in this report, the recommendations will address Saathban Service design and overall business model.

CALL FOR CONTRIBUTION

Saathban welcomes comments, corrections, and collaboration on any of the sections above. Readers who would like to contribute case studies, regional data, institutional profiles or policy analysis are invited to write to hello@saathban.org. A revised and expanded edition of this report, incorporating feedback received, is planned for publication in due course.

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END OF FIRST DRAFT

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www.saathban.com · hr@saathban.com